

Web GUI, Analytics and Clinical Presentation Coding Handoff

Expanded edition with page examples, patient-to-clinician design, chart contracts and implementation architecture.

For senior engineers, product leaders, data engineers, clinical workflow reviewers, security reviewers and implementation partners.

This document preserves the original GUI research handoff and adds the complete target web atlas, visual page examples, role analytics, projection contracts, APIs, components, test requirements and build sequence.

Development prototype. Fabricated-data design examples. This document does not approve clinical use or change Steady's deterministic safety rules.

ACTION FIRST. MEANING SECOND. EVIDENCE THIRD. RAW RECORDS LAST.

Design from the decision backward. Build from the evidence forward.

How to use this handoff

Read the decision surface before the component list. Read the data contract before the page implementation.

Part I contains the original 33-page GUI and competitor research handoff, beginning with its handoff map and continuing through routes, screen specifications, components, tokens, endpoints, tests and delivery rules.

Part II is the new coding annex. It contains 80 proposed web screens, 20 page examples, 22 chart screens, the organized patient-to-clinician presentation model, role projection architecture and the coding plan.

The annex is additive. When the documents differ, the stricter safety, privacy, evidence, denominator or release requirement controls.

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Combined handoff contents

Front matter	Pages 1-4
Purpose, reading map, scope and build map	
Original GUI and decision-surface handoff	Pages 5-36
Current state, competitor evidence, target roles, screens, components, routes, tests and delivery	
Expanded coding annex	Pages 37-101
80-screen atlas, patient-to-clinician model, page examples, analytics, architecture, build and release	

The final file uses one continuous footer and page count.

Build map

The handoff connects user decisions to architecture and release evidence.

1. Start with the screen atlas and select a vertical slice.
2. Implement the event, policy and projection contract before the page.
3. Compose the page from shared decision and evidence components.
4. Implement every loading, stale, partial, denied and failed state.
5. Add action, audit, accessibility, authorization and replay tests.
6. Close the slice only when the visible decision, evidence path, failure path and audit path pass.

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Design from the decision backward. Build from the evidence forward.

Handoff map

This document starts with what Steady looks like now, compares six public product interfaces, then specifies the target role views, presentation contracts, gates, components, routes, tests, and build order.

Current state	Sections 1-3	Decision, scope, public routes, iOS screens, safety copy, and contrast
Market patterns	Sections 4-5	NeuroFlow, Blueprint, Greenspace, Osmind, Spring Health, and Eleos
Target product	Sections 6-10	Roles, navigation, presentation layers, gates, and screen wireframes
Build system	Sections 11-17	Components, tokens, charts, states, audit behavior, endpoints, web and iOS
Delivery	Sections 18-23	Build order, backlog, release criteria, tests, telemetry, and final rules

Build principle

Design from the decision backward. Build from the evidence forward.

1. Executive decision

Steady does not need a new data foundation before it can become easier to use. It needs a presentation layer that turns its existing foundation into clear decisions.

The system already records events, scores measures, runs deterministic gates, validates cited summaries, logs review actions, and preserves audit history. The current screens often expose those parts as separate facts. The next GUI should join them into one repeated pattern:

What changed? Why does it matter? What can this user do now? What will be recorded when they act?

The product direction is a role-based quiet cockpit. It should feel calm without hiding urgency, data-rich without leading with tables, and trustworthy without forcing a user to read implementation details before taking the next safe step.

The core design rule is:

Action first. Meaning second. Evidence third. Raw records last.

This does not mean hiding evidence. It means placing evidence one layer below the decision surface, with timestamps, sources, policy versions, and review status available on demand.

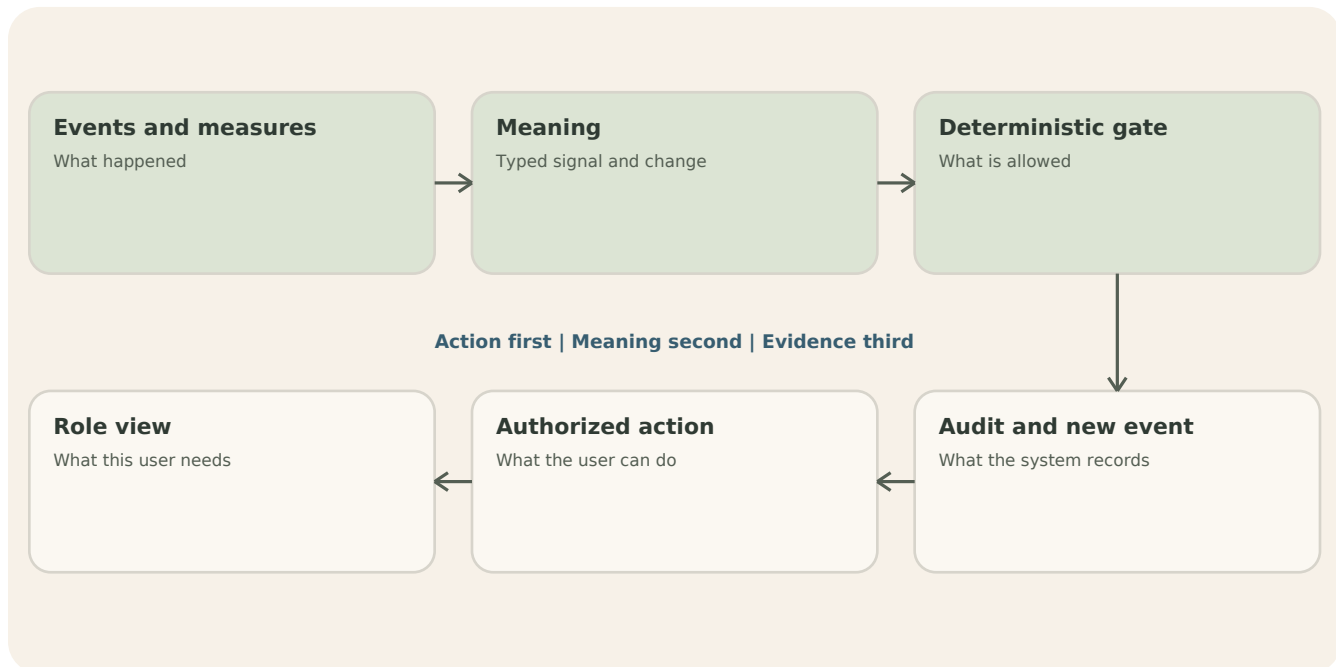
What should change

- Replace the member Home inventory with a focused Today screen containing one primary action and no more than two secondary actions.
- Replace clinician dashboards led by charts with a work queue led by priority, reason, owner, freshness, and next action.
- Give each person record a current-state header, a “since last review” summary, an action area, and a unified timeline.
- Present gates as understandable states with a reason, a safe alternative, and an explicit path forward.
- Separate deterministic decisions, human decisions, and model-generated suggestions visually and in the API contract.
- Give organization and payer users aggregate views built around outcomes, engagement, operations, and denominators. Do not reuse the clinician person view.
- Keep audit, replay, citations, and rule details available through drawers and dedicated views.

What should not change

- Deterministic safety gates remain authoritative.
- A model cannot promote itself into a governing role.
- Corrections append and supersede. They do not erase history.
- Safety stops cannot be relaxed through ordinary override controls.
- Grounding and crisis resources remain reachable even when a write, subscription, sync, or service fails.

- Tenant, role, consent, and minimum-necessary checks occur before a presentation object reaches the client.



2. Scope and evidence boundary

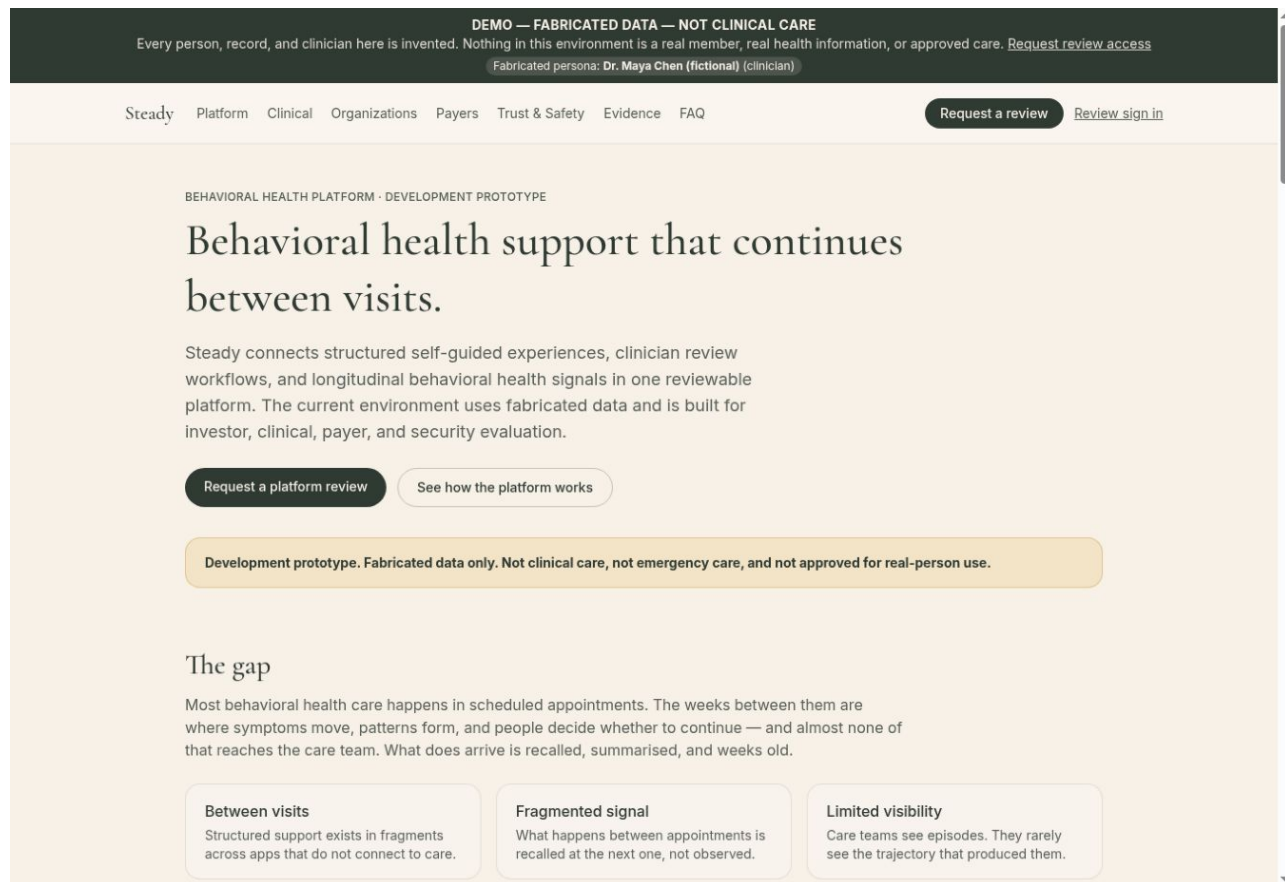
This handoff is based on four evidence sets.

- 1 The public Steady web environment was inspected route by route on August 28, 2026. Fourteen public routes were reviewed, including the institutional, trust, evidence, sign-in, and crisis surfaces.
- 2 The current Steady web repository `tprawlins-lang/Emdr` was reviewed at commit `c39447a715e92b4800889c7afd6d015e39457c8f`, dated August 27, 2026. All 53 page routes were inventoried, with close source review of the member dashboard, review gateway, clinician dashboards, caseload, person records, alert trail, autonomous console, BLS oversight, testing, and audit views.
- 3 The Steady iOS repository `tprawlins-lang/Steady-ios` was reviewed on its `main` branch at commit `f9a2645fc57135dfe1c7ee61c0e327a4ddc35a0d`, dated July 29, 2026. Every SwiftUI view and session state was inventoried.
- 4 Public product pages, help material, app listings, and product screenshots were reviewed for NeuroFlow and five comparable systems: Blueprint, Greenspace, Osmind, Spring Health, and Eleos.

The public deployment does not expose review credentials, so the locked reviewer paths were not directly observed in that deployment. Their current structure, content order, controls, states, and route relationships were confirmed from the current source. Public pages were both visually inspected and checked against source. This distinction matters because a source-confirmed layout can establish what the screen renders, but it cannot substitute for formal usability testing with each role.

Competitor findings are also bounded. Public screenshots often show selected paths and may be marketing composites. They support layout and information-pattern findings, but they do not prove every state, permission rule, or current production detail.

3. Current Steady visual system



Current Steady public home, inspected August 28, 2026. The visual language is calm and credible, but the page behaves like a long evidence document rather than an application surface.

The public site uses an ivory and linen base, dark green text, serif display headings, restrained borders, rounded cards, and muted semantic colors. Most institutional content sits in an 896-pixel column. The home route widens to 1,024 pixels. This gives the brand a composed, human tone.

The strongest current visual choices are worth keeping:

- Ivory and linen surfaces reduce glare and separate Steady from generic blue healthcare software.
- Dark ground text provides strong readability on light surfaces.
- Serif headings give the product a calm identity, while the sans-serif body font keeps dense copy readable.
- Rounded cards and generous spacing lower the emotional temperature.
- Safety messages use words as well as color.
- The iOS session player keeps pause, stop, grounding, and crisis paths within reach.

The main problem is not the brand. It is information structure.

3.1 Public route inventory

Route	Current visual form	What it asks the reader to do	Main issue
/	Long institutional landing page, cards, badges, one status table	Understand the entire platform	Too many claims and system facts compete before role selection
/platform	Narrow explanatory page	Learn the three platform layers	Sequence is described, not shown as a working interface
/clinical	Workflow copy and policy table	Judge clinical accountability	Describes actions without a visible work queue or person record
/organizations	Operating-model narrative	Consider a pilot	No aggregate operating dashboard is shown
/payers	Measurement narrative	Consider an evaluation	No denominator-led payer view is shown
/trust	Very long control and gap inventory	Review security posture	Correctly detailed, but difficult to scan and prioritize
/evidence	Evidence-status narrative	Separate method evidence from Steady evidence	Good claims discipline, limited visual structure
/faq	Long grouped question list	Resolve objections	Dense and repetitive for return visitors
/about	Position and stage narrative	Understand why Steady exists	Reads as a document, not a role entry point
/request-review	Review-path selection	Request access	Clear, but no preview of the guided route after approval
/login	Centered sign-in card	Enter the review environment	Clean and focused
/crisis	Narrow support page	Reach immediate help	Clear, compact, action led
/terms	Legal text	Review demo terms	Appropriate document form
/privacy	Legal text and one table	Review demo data handling	Appropriate document form

The public site should remain information-rich, but the home route should send each role to a short, visual preview of its actual workspace. Trust, evidence, terms, and privacy can remain document-like because their job is reference and review.

3.2 Current web application inventory

The current web repository contains 53 page routes. It already implements most of the raw material needed for a strong role-based product, but the material is distributed across long pages and overlapping consoles.

Surface group	Current routes	Current visual form	Main GUI issue
Review entry	/demo, /demo/[path], /login	Boundary notice, path cards, access code, fabricated persona cards	Clear entry sequence, but it does not preview the workspace or task the reviewer will enter
Member core	/dashboard, /check-in, /companion, /ground, /paths, /measures	Long stacked dashboard, cards, charts, libraries, forms	The dashboard mixes status, next steps, five practices, measures, paths, plan, and full module catalog
Member setup and controls	Onboarding, screening, consent, billing, account, voice, memory, session settings	Long vertical forms and repeated cards	Strong disclosure, weak sense of place and progress across the full setup sequence
Specialist entry	/clinician and /clinician/member/[id]	Alerts, review work, member table, outcome charts, long detail record	This older entry overlaps with the newer clinical console and creates two mental models

Surface group	Current routes	Current visual form	Main GUI issue
Clinical work	<code>/clinician/clinical,</code> <code>/clinician/clinical/[id],</code> <code>/clinician/alerts/[id]</code>	Alert list, need-ordered caseload, cited summary, timeline, override, audit	Good evidence discipline, but decisions, evidence, corrections, and audit compete in one vertical flow
Governance and validation	<code>/clinician/autonomous,</code> <code>/clinician/bls,</code> <code>/clinician/testing,</code> <code>/clinician/audit</code>	Simulation forms, engine outputs, sign-off registers, state tables, export controls	Designed like technical review documents, with limited persistent navigation and high scanning cost
Organization and payer	<code>/organizations,</code> <code>/payers</code>	Public narrative pages	No working aggregate workspace exists yet

The current web application has several strong building blocks:

- The clinical caseload orders people by need and shows reasons instead of presenting an unexplained rank.
- Alerts show band, due time, overdue state, detail, audit trail, and a required documented resolution.
- The clinical person summary suppresses unsupported claims and shows evidence identifiers for rendered claims.
- Model-produced timeline entries are visually marked, and corrections append a superseding record.
- The interface states that ordinary overrides can relax pacing only, never a safety stop.
- The review gateway clearly labels fabricated data, purpose scope, and write-capable roles.

The next GUI should preserve those controls while removing duplication. `/clinician` and `/clinician/clinical` should become one workspace. `/clinician/member/[id]` and `/clinician/clinical/[id]` should become one person record with tabs or anchored sections. Governance consoles should sit under one Review and Controls area instead of appearing as equal top-level destinations beside daily clinical work.

3.3 Current iOS route and state inventory

The iOS application already contains far more than one dashboard. Its present screens fall into six groups.

Group	Current views and states	Current GUI pattern
Entry	Login, signup, local-only mode	Centered form and stacked cards
Program entry	Subscription, consent, fit screener, cooldown, five baseline instruments, seven-step profile	Long vertical forms and repeated soft cards
Main shell	Home, Check-in, Companion, Ground, Settings	Five-tab member shell with persistent SOS button
Daily support	Breathing, meditation, movement, sleep, learning	Repeated libraries of cards followed by simple players
Program	Module list, module detail, locked state	Ordered rows, tier badge, step list, single start action
Session	Intro, optional preparation, instruction, distress rating, bilateral sets, grounding pause, hard stop, completion, trigger capture	State machine with large single-purpose actions

3.4 Current member Home

The current Home screen stacks:

- Greeting
- Daily check-in card
- Optional Autopilot card

- Breathe card
- Meditate card
- Move card
- Sleep card
- Learn card
- Distress trend
- Twelve module rows
- Wellness disclaimer

This is a content catalog. It tells the member everything Steady can do, but it makes the member decide what matters now. On a hard day, that choice load is exactly what the system should reduce.

3.5 Current check-in result

The check-in correctly runs a deterministic decision. The result card gives a headline and detail. Except for a crisis result, it does not reliably carry the member into the recommended activity. The member must leave the result and find the right tool.

The result should become a decision card with:

- Today's state in plain language
- The main reason, without exposing sensitive scoring internals by default
- One recommended action
- One safe alternative
- How long the state applies
- A “why this?” drawer
- A crisis or support path that never depends on a successful save

3.6 Current progress display

The current chart shows ending distress across recent sessions. It lacks dates, baseline, goal or expected range, event annotations, confidence or missing-data handling, and a plain-language interpretation. “Lower is calmer” explains direction, but not meaning.

A trend should never stand alone. It should pair with:

- Comparison period
- Current value and change
- Plain-language interpretation
- Relevant event markers
- Missing or stale data notice
- A link to the underlying entries
- An accessible data table

3.7 Current gates and locks

Locked modules show “Not available right now” and a reason. This is a good base. The next version should distinguish:

- Temporarily limited by today's check-in
- Waiting for a named human review
- Locked by program sequence
- Unavailable because of consent or profile state
- Stopped by a safety rule
- Unavailable because data is stale or missing
- Unavailable because the service cannot confirm access

Those states are not interchangeable. They must not share one lock icon and one generic message.

3.8 Current safety language that must be corrected

The iOS hard-stop screen currently says a care team has been notified. The public demo says no one monitors the environment and no care team is assigned.

The GUI must never claim notification from an attempted write. Notification has its own state machine:

State	Member-safe language
Not configured	“This activity is not monitored. Use the support options below.”
Queued	“Steady is trying to send an alert. Do not wait for a response if you need help now.”
Delivered	“An alert was delivered to your care team at 2:14 PM. Response times vary.”
Acknowledged	“Jordan Lee acknowledged this alert at 2:19 PM.”
Failed	“Steady could not deliver the alert. Use the support options below now.”

Only a delivery receipt can support “delivered.” Only a separate acknowledgment event can support “acknowledged.”

3.9 Accessibility issue in the current color tokens

The main text colors perform well. Several accent colors do not meet the 4.5:1 contrast target for small text on ivory or linen.

Current token	Contrast on ivory	Use now	Required change
ground	10.54:1	Primary text	Keep
olive	6.03:1	Secondary text	Keep
support	5.24:1	Danger text	Keep, test every background
mistDeep	4.18:1	Lock and secondary accents	Do not use for small text on ivory
safeDeep	3.20:1	Success text	Replace for text
sageDeep	2.34:1	Links, labels, icons	Restrict to large non-text decoration or replace
clay	1.96:1	Selected backgrounds and accents	Background only

[W3C WCAG 2.2 contrast guidance](#) supports a 4.5:1 minimum for normal text. Focus indicators also need clear size and contrast, and controls should meet the WCAG 2.2 target-size rules. [W3C focus guidance](#) and [W3C target-size guidance](#) should be part of the release checklist.

4. What NeuroFlow and five comparable systems show

The products do not share one visual style. They do share a small set of information patterns that reduce the distance from data to action.

4.1 NeuroFlow

NeuroFlow uses different visual forms for the member, care team, and population viewer.

- The member experience centers tasks, activities, trackers, assessment prompts, progress, and rewards. Public app material lists Home, To Do, Discover, Crisis, and Profile as the primary navigation model.
- The tracker view lets a member switch among mood, sleep, stress, pain, and pain impact across selected time windows.
- Public provider images show a person summary with assignment completion, questionnaires, engagement, and alerts.
- Longitudinal measure screens use time-series charts, severity meaning, and suggested follow-up.
- Care-team dashboards support individual and population review, including at-risk cohorts and outliers.
- Journeys can be selected by the member, assigned by a care manager, or selected based on severity.

The main lesson for Steady is not gamification. It is role separation. The member sees manageable activities. The care team sees exceptions and follow-up. The enterprise viewer sees program performance.

Sources: [NeuroFlow digital behavioral health](#), [NeuroFlow solutions](#), [NeuroFlow measurement-based care](#), and [Onward by NeuroFlow on the App Store](#).

4.2 Blueprint

Blueprint's public dashboard image uses a strong two-column opening view.

- The left column is a notification queue. Each item names the person, the event, risk status, recency, and a Review action.
- The right column summarizes active, overdue, and high-risk clients.
- The product positions the dashboard as a way to get caught up before a session, not as a place to study raw records.
- The client app uses due assessments, daily check-ins, and personal trends.

The main lesson for Steady is that a clinician home should open on work, not charts. A chart belongs inside the person record or behind a population question.

Sources: [Blueprint guided measurement-based care](#), [Blueprint for Clients on the App Store](#).

4.3 Greenspace

Greenspace uses a restrained clinical dashboard with three visible levels.

- Top navigation separates Patients, Therapists, and Dashboard.
- The dashboard combines longitudinal symptoms and therapeutic alliance, number of patients, assessment completion, and a recent-activity heatmap.
- The patient list gives a progress snapshot before the user opens a detailed record.
- Organization views add adoption, engagement, and clinical-outcome benchmarks.

The main lesson for Steady is to keep population views small in number and stable in meaning. Adoption, engagement, outcomes, and activity are enough top-level groups. Each should drill into a defined cohort.

Sources: [Greenspace platform](#), [Greenspace benchmarks dashboard](#).

4.4 Osmind

Osmind uses an EHR-style shell organized around patient charts, schedule, and reports.

- A persistent navigation rail keeps high-frequency practice tasks available.
- Patient charts act as the center of the clinician workflow.
- The patient app extends the treatment plan into measures, journaling, messaging, medication reminders, and insurance documents.
- Public product material states that symptom-threshold alerts and automated rating scales sit within the charting workflow.

The main lesson for Steady is to attach between-visit data to the person's care context. A signal without the care plan, last review, ownership, and communication history creates more work.

Sources: [Osmind mental health EHR](#), [Osmind patient portal](#), [Osmind Care on the App Store](#).

4.5 Spring Health

Spring Health's current public materials show role-specific continuity.

- The provider home opens with today's sessions, risk tags, join or review actions, notes, invoices, and caseload tasks.
- The To Do list orders unfinished work by urgency.
- The person view brings anxiety, depression, sleep, clinical insights, notes, and session context into one continuous record.
- The member experience leads with the next step, such as booking another session.
- Employer views convert aggregate results into a small set of benchmarked insight cards.

The main lesson for Steady is to connect continuity with the next step. A member, provider, and organization can see different forms of the same underlying record without sharing the same dashboard.

Sources: [Spring Health Guide](#), [Spring Compass provider overview](#), [Spring Health member platform](#).

4.6 Eleos

Eleos focuses on work completion inside the provider's existing environment.

- The clinician flow is capture, draft, review, edit, and submit.
- Suggested note content is visibly reviewable before it becomes final.

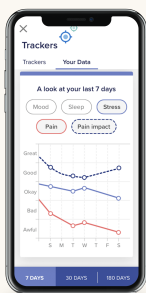
- Session analytics surface selected care and documentation signals rather than presenting a transcript as the main view.
- Leadership reporting highlights a few operating metrics, such as note volume, completion time, hours saved, activation, and active use.

The main lesson for Steady is that model output should enter a human task, not become a free-standing truth card. Suggested content needs a status, evidence, editable boundary, and final human action.

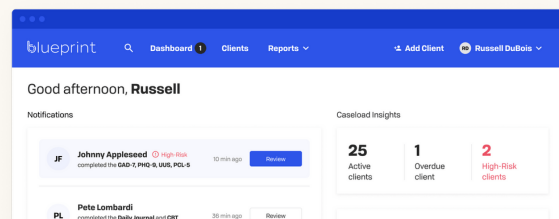
Sources: [Eleos documentation product](#), [Eleos community behavioral health](#).

5. Competitor screen evidence

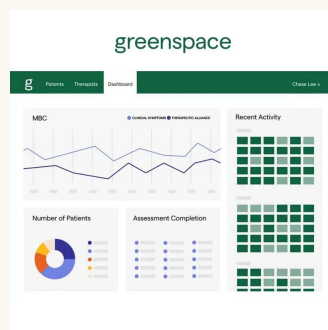
The screenshots below are public product images. They illustrate layout patterns, not verified end-to-end behavior.



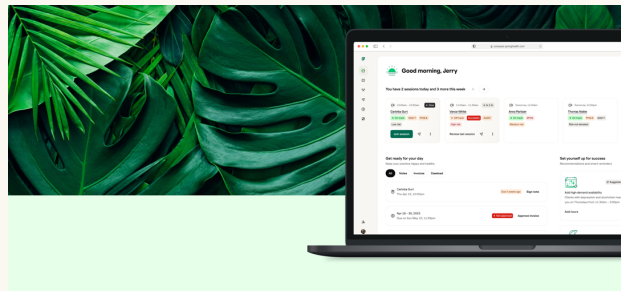
NeuroFlow member tracker
Capterra public product screenshot



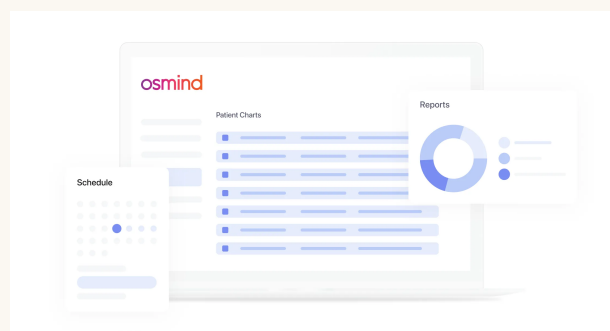
Blueprint clinician queue
Blueprint public product image



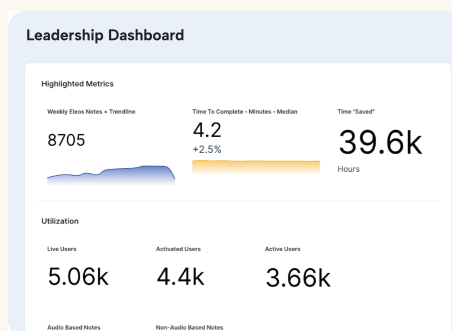
Greenspace aggregate dashboard
Qualifacts public product image



Spring provider day view
Spring Health public product image



Osmind chart, schedule, and reports
Osmind public product image



Eleos leadership dashboard
Eleos public product image

Images are included for comparative analysis. Product interfaces may have changed after the public image was published.

Pattern summary

Pattern	NeuroFlow	Blueprint	Greenspace	Osmind	Spring	Eleos
Member next step	Strong	Strong	Moderate	Care-plan led	Strong	Not primary
Clinician work queue	Strong	Strong	Patient-list led	Chart and schedule led	Strong	Documentation led
Individual trend	Strong	Strong	Strong	Strong	Strong	Session analytics
Aggregate dashboard	Strong	Limited public view	Strong	Reports	Strong	Strong operations view
Human review of model output	Limited public detail	Current AI workflow	Not primary	Current AI workflow	AI insights with oversight	Core pattern
Public GUI lesson for Steady	Separate roles	Put work first	Stable metric groups	Keep care context attached	Connect continuity to action	Make AI a review task

6. Target experience: one platform, different questions

Steady should not build one dashboard and hide columns by role. Each role has a different first question.

Role	First question	Primary object	First action
Member	“What is the safest useful thing for me now?”	Today plan	Start, choose another safe option, or get support
Clinician	“Who needs my attention and why?”	Work item	Review, contact, correct, approve, or defer with reason
Care manager	“What needs coordination today?”	Owned queue	Outreach, assign, close loop, or escalate
Supervisor	“Where is work backing up or varying?”	Team queue and operational metric	Reassign, coach, inspect, or change workflow
Organization leader	“Are people engaging and improving, and where are the gaps?”	Cohort metric	Drill down, compare, and plan an operating response
Payer or value-based lead	“Who is reached, what changes, and what does it cost?”	Defined population measure	Review performance and methodology
Security or compliance reviewer	“Who accessed or changed what, and can the record be trusted?”	Audit event and control state	Investigate, attest, export, or remediate

6.1 Shared product grammar

Every role should see the same six status concepts, expressed at the right level.

- 1 State: What is true now.
- 2 Change: What moved since the relevant comparison.
- 3 Reason: What evidence and rule produced the state.
- 4 Action: What this role is allowed to do.
- 5 Owner: Who is responsible for the next step.
- 6 Freshness: When the data and decision were last updated.

This grammar should live in API contracts and component props, not only in copy guidelines.

7. Target information architecture

7.1 Member navigation

Replace the current five tabs with:

Tab	Purpose	Current content moved here
Today	One daily plan, state, and recent completion	Check-in, Autopilot, immediate next action
Companion	Conversation and suggested actions	Companion, memory controls through profile
Practice	Browse safe activities and program modules	Breathe, meditate, move, sleep, Learn, modules
Progress	Understand change over time	Session trend, measures, milestones, timeline
Support	Immediate regulation and human help	Ground, SOS, crisis resources, safety plan

Profile and settings move to the avatar in the top-right corner. SOS remains reachable from every tab, but it should appear as a labeled “Support” control rather than a floating acronym alone. If the red floating control remains, its accessible and visible label should explain its function.

7.2 Clinician navigation

Route	Purpose
/clinical/work	Prioritized work queue
/clinical/people	Searchable caseload with compact state summaries
/clinical/people/[id]/overview	Current state, change, actions, care context
/clinical/people/[id]/timeline	Unified longitudinal timeline
/clinical/people/[id]/measures	Measures with meaning, ranges, and event markers
/clinical/people/[id]/plan	Goals, assignments, restrictions, and ownership
/clinical/reviews	AI summaries, gates, corrections, approvals, and overrides awaiting review
/clinical/reports	Caseload and workflow reporting
/clinical/audit	Role-scoped audit history

7.3 Organization navigation

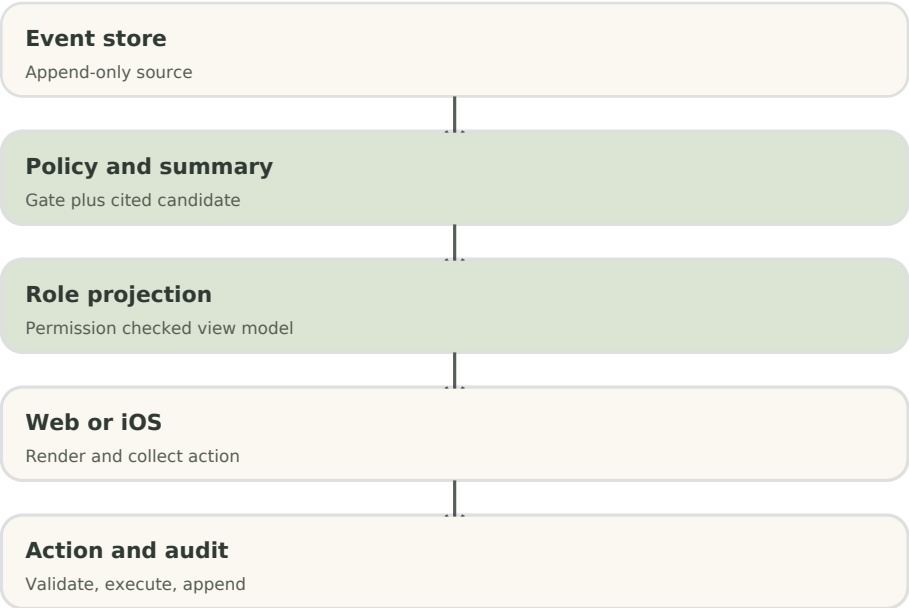
Route	Purpose
/organization/overview	Outcomes, engagement, operations, and access at a glance
/organization/engagement	Eligibility-to-active funnel, retention, completion
/organization/outcomes	Measure change, reliable improvement, stratified cohorts
/organization/operations	Queue time, outreach, review completion, workload
/organization/equity	Access and outcome differences with privacy controls
/organization/governance	Policy versions, review coverage, data quality, exports

7.4 Payer navigation

Route	Purpose
/payer/overview	Population reach, engagement, outcomes, and utilization
/payer/populations	Eligible, invited, enrolled, active, supported, improved
/payer/measurement	Definitions, denominators, time windows, exclusions
/payer/utilization	Approved cost and use measures when available
/payer/methodology	Cohort logic, attribution, missingness, and caveats

8. Presentation architecture

The UI should never build clinical meaning by joining raw event arrays in React or SwiftUI. The server should publish role-scoped presentation objects.



8.1 Layer responsibilities

Layer	Responsibility	Must not do
Event store	Preserve what happened	Decide what a user should see
Normalization	Convert events into typed signals	Apply role permissions late
Deterministic policy	Produce gates, bands, and allowed actions	Generate final user copy
Summary pipeline	Produce cited candidate statements	Bypass validation or human review state
Presentation service	Build role-scoped view models	Recalculate domain policy in the client
Client	Render, collect an authorized action, show state	Infer safety, fabricate freshness, or claim delivery
Action service	Validate and execute the action	Trust a client-provided owner, role, or gate
Audit service	Append actor, action, evidence, and result	Depend on display text as the source of truth

8.2 Shared presentation contract

```
type SourceKind = "event" | "measure" | "policy" | "review" | "integration";

interface EvidenceRef {
  id: string;
  kind: SourceKind;
  occurredAt: string;
  recordedAt: string;
  label: string;
  href?: string;
}

interface Freshness {
  computedAt: string;
  newestEvidenceAt: string;
  staleAfter: string;
  state: "fresh" | "aging" | "stale" | "unknown";
}

interface ReasonItem {
  code: string;
  label: string;
  detail?: string;
  evidence: EvidenceRef[];
}

interface AllowedAction {
  id: string;
  label: string;
  kind: "navigate" | "review" | "contact" | "approve" | "correct" | "override" | "export";
  href?: string;
  confirmation: "none" | "standard" | "high_risk";
  disabledReason?: string;
}

interface DecisionSurface {
  id: string;
  state: string;
  headline: string;
  explanation: string;
  change?: { label: string; direction: "better" | "worse" | "mixed" | "stable" | "unknown" };
  reasons: ReasonItem[];
  actions: AllowedAction[];
  owner?: { id: string; displayName: string; role: string };
  freshness: Freshness;
  policy?: { id: string; version: string };
  review?: { state: "not_needed" | "pending" | "approved" | "corrected" | "rejected";
    reviewedAt?: string };
}
```

The server creates the `DecisionSurface`. The clients render it. The clients may choose a mobile or desktop layout, but they may not change `state`, `reasons`, `allowed actions`, or `review state`.

8.3 Projection tables or caches

Create read-optimized projections so the screen does not assemble state from dozens of queries.

Projection	Key	Used by
member_today_projection	person, local date	Today screen
member_progress_projection	person, period	Progress screen
clinical_work_item_projection	organization, work item	Clinician and care-manager queues
person_status_projection	organization, person	Person header and caseload rows
person_timeline_projection	organization, person, cursor	Timeline

Projection	Key	Used by
gate_decision_projection	person, module, decision version	Member lock and clinician review
organization_metric_snapshot	organization, cohort, period	Organization views
payer_metric_snapshot	contract, population, period	Payer views
notification_delivery_projection	notification attempt	Member and staff notification copy

Each projection must include `computed_at`, `source_watermark`, `policy_version`, and `tenant_id`. A projection is disposable. The event and review records remain authoritative.

9. Gating as a visible, understandable state

9.1 Member-facing gate states

State	Visual treatment	Member message	Primary action
Open	Neutral or safe icon with text	“This is available today.”	Start
Caution	Warm neutral banner	“Go gently today because your activation is higher than usual.”	Start gentler option
Limited	Amber banner with reason	“Processing is paused today. Grounding and regulation remain open.”	Start grounding
Review needed	Blue-gray banner with owner and timing	“A specialist review is needed before this opens.”	See what happens next
Safety stop	High-contrast support banner	“This session stopped because continuing was not the safe choice.”	Ground now or get help
Unknown	Neutral warning	“Steady cannot confirm access right now.”	Use always-open tools and retry

Never rely on green, amber, or red alone. Every state needs an icon, a label, and one-sentence meaning.

9.2 Clinician-facing gate drawer

The clinician sees more detail without exposing raw implementation data in the first row.

Gate review

Person: Maya R. | Decision effective 10:42 AM

LIMITED

Processing paused today

Why

Activation rose; sleep low

Policy

daily-gate v14.3

Evidence

Check-in 10:41 AM | Measure 3 days ago | Prior gate Open

Allowed actions

Approve pacing | Correct reason | Contact

Review and record action

The drawer must show:

- Current decision and effective time
- Rule and version
- Human-readable reasons
- Evidence references
- What the clinician can approve, correct, or override
- What cannot be overridden
- Prior decision and change
- Member-safe copy preview
- Action confirmation and resulting audit record

9.3 Model-generated content

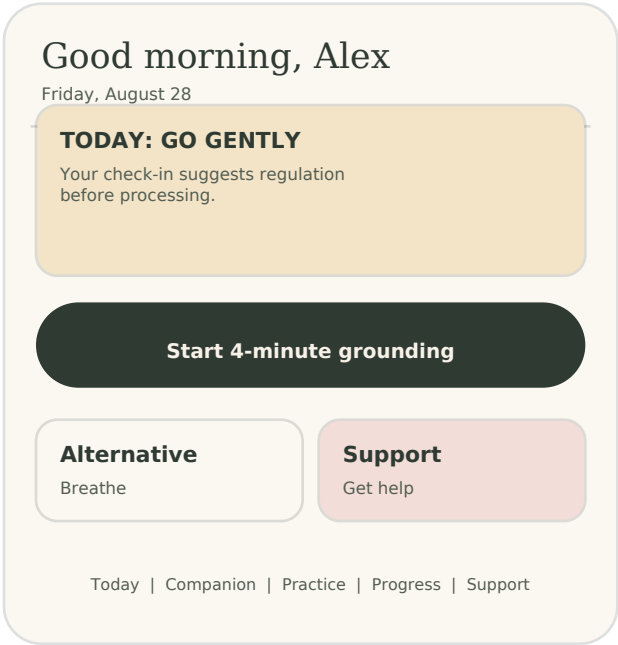
Use four distinct labels:

State	Label	Allowed placement
Candidate	"AI draft"	Review queue only
Validated	"Evidence checked"	Review queue and person view, still not approved
Human approved	"Reviewed by [name]"	Clinical record and downstream summary
Corrected	"Corrected by [name]"	Clinical record with superseded draft available in history

Do not use a sparkle icon as the only sign that text came from a model. Do not style model output like a system fact.

10. Screen specifications

10.1 Member Today



Purpose: remove choice load and connect the check-in to the next safe action.

Above the fold:

- Greeting and local date
- Data freshness only when stale or delayed
- Today state card
- One primary action with expected time
- One sentence explaining why
- Support control

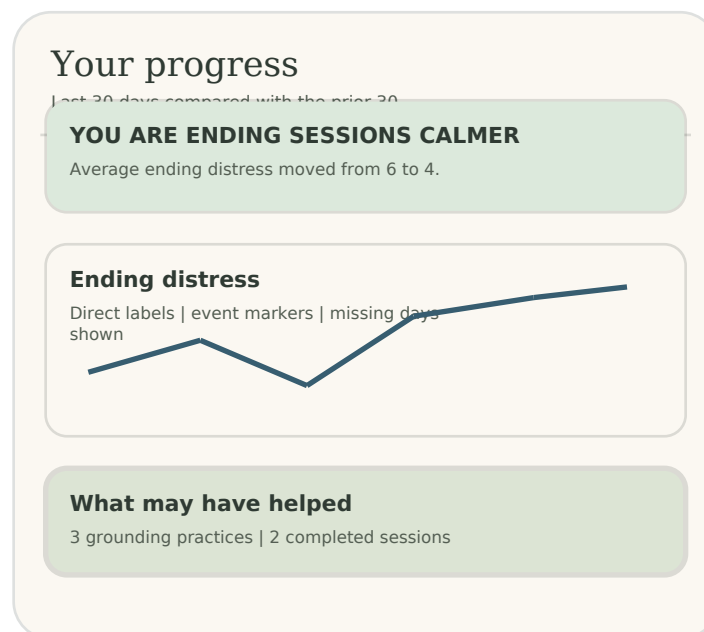
Below the fold:

- Up to two safe alternatives
- Completed activity and gentle progress
- Optional “why this plan?” drawer
- Upcoming care or review item if one exists

Rules:

- No full module catalog on Today.
- No paywall card above the member's safe action.
- If the check-in is due, it becomes the primary action.
- If a check-in returns Limited or Safety stop, the next action updates in place.
- If data is stale, say what is stale and preserve always-open options.

10.2 Member Progress

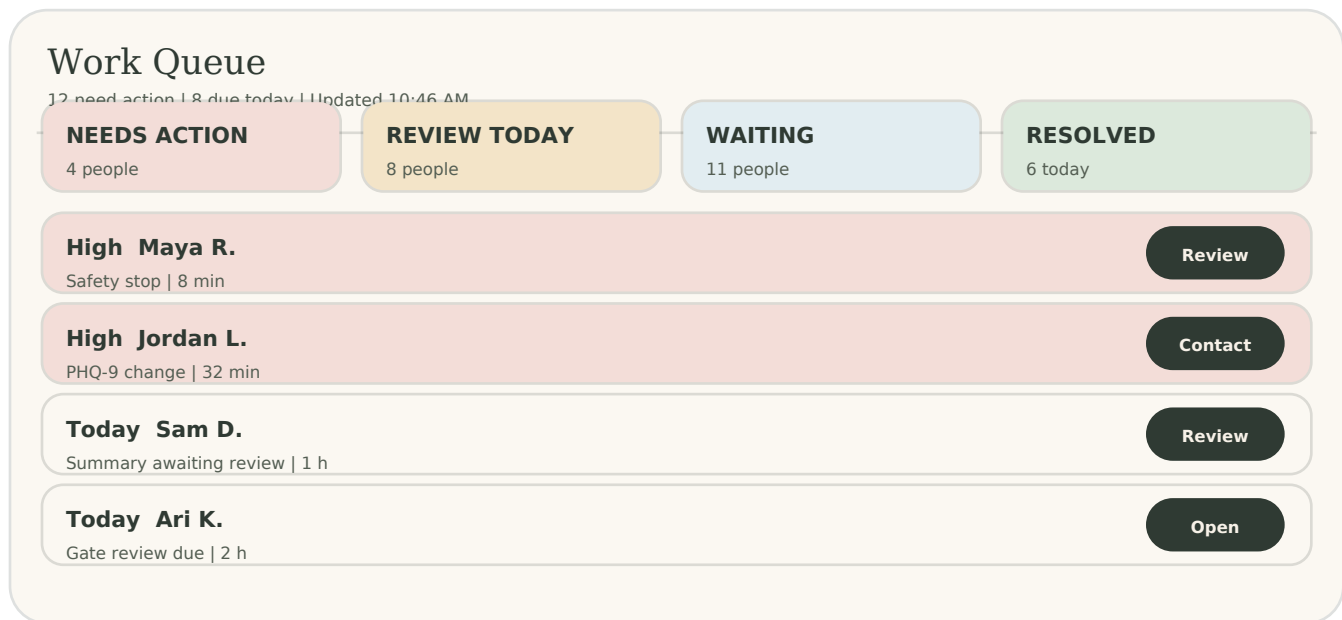


Opening order:

- 1 Plain-language change statement.
- 2 Current period compared with prior period.
- 3 One primary trend.
- 4 Event markers that may explain movement.
- 5 Measure cards and session history.
- 6 Full data table and source history.

Do not merge measures with different scales into one unlabeled line chart. If several signals matter, show small multiples with the same time range or a summary index whose construction is disclosed.

10.3 Clinician Work Queue



Required row fields:

- Priority band and label
- Person display identity
- Reason for appearing
- Change since last review
- Most recent evidence time
- Current owner
- Due or response target
- One row action

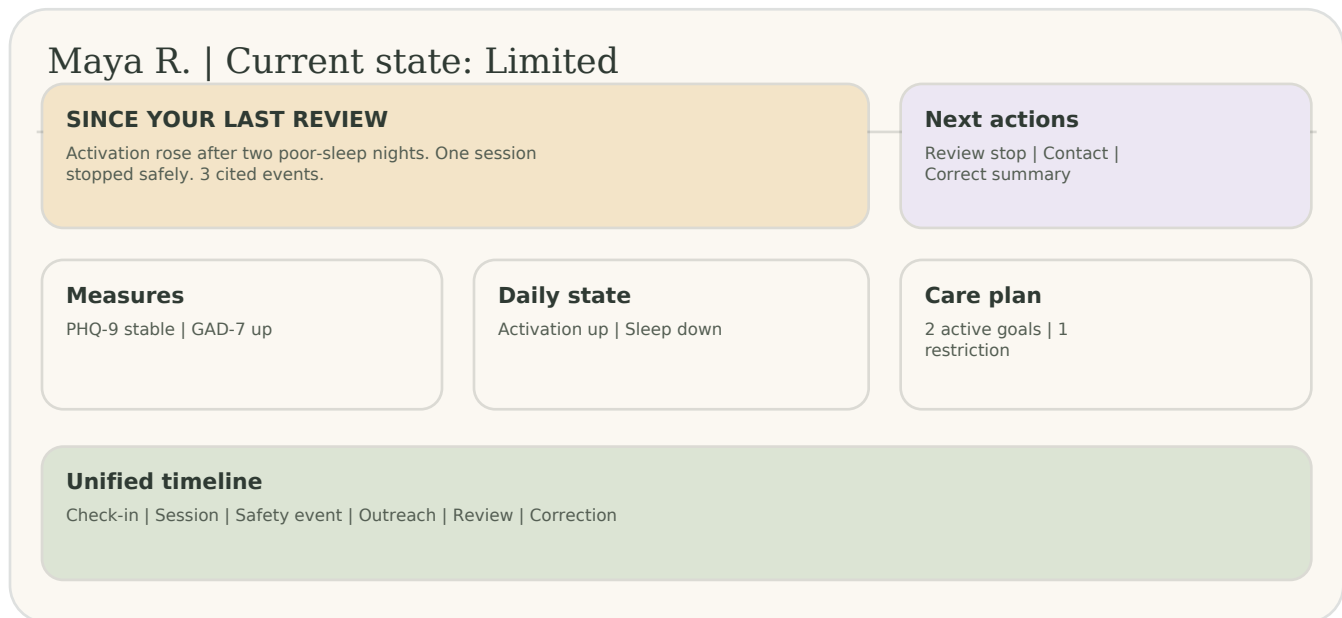
Queue groups:

- Needs action now
- Review today
- Waiting on member

- Waiting on staff or integration
- Recently resolved

Duplicate alerts for the same person and reason should collapse into one work item with an event count. AHRQ describes alert fatigue as desensitization caused by too many alerts, especially when most do not require action. [AHRQ alert fatigue primer](#). Steady should measure alert volume, duplicate suppression, time to action, and the share of alerts that result in a meaningful action.

10.4 Clinician Person Overview



Sticky header:

- Display identity
- Care relationship and owner
- Current state and freshness
- Consent or sharing boundary
- Contact action
- More menu for lower-frequency actions

Main content:

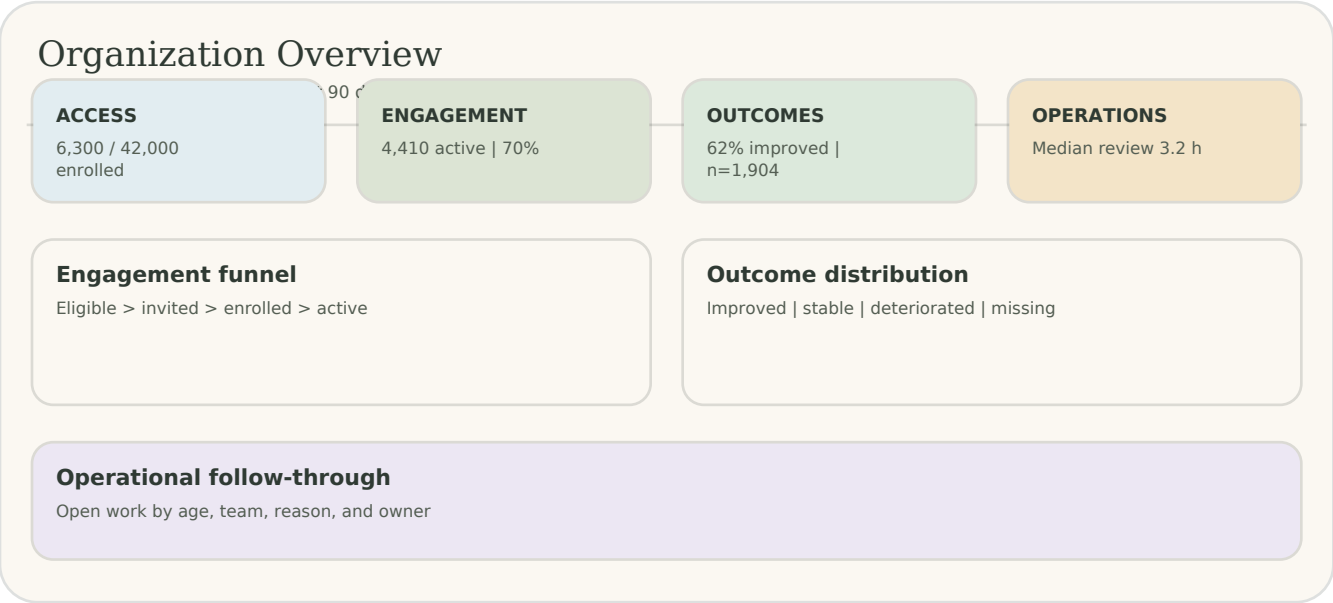
- “Since your last review” summary with citations
- Active work and pending reviews
- Measures and daily-state trends
- Current care plan, restrictions, and member preferences
- Unified timeline

Right rail:

- Allowed actions
- Safety plan access

- Assigned team
- Integration or sync warnings

10.5 Organization Overview

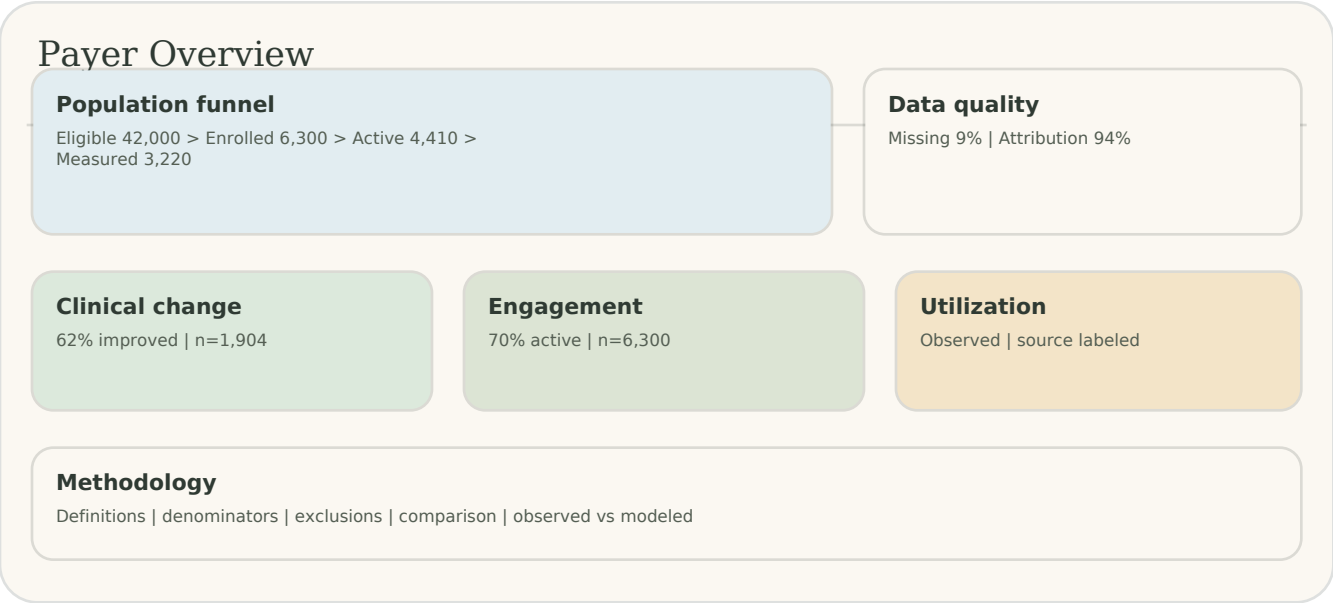


Use four stable sections:

- 1 Access: eligible, invited, enrolled.
- 2 Engagement: active, task completion, retention.
- 3 Outcomes: measure change, reliable improvement, deterioration.
- 4 Operations: queue time, reviews, outreach, unresolved work.

Every metric card must show numerator, denominator, time range, change, and cohort filter. A percentage without its denominator is not acceptable.

10.6 Payer Overview



The payer screen should not resemble a clinician dashboard. It should open with a population funnel and a small number of contract-defined measures.

Required disclosures:

- Eligible population definition
- Attribution rule
- Measurement window
- Minimum enrollment duration
- Missing-data rate
- Exclusions
- Comparison group, if any
- Whether a value is observed, modeled, or illustrative

No person-level drilldown should exist unless contract, permission, minimum-necessary purpose, and policy explicitly support it.

10.7 Trust and Audit

The trust workspace should convert the current long control page into a working review surface.

View	First information	Primary action
System health	Current control state and failures	Inspect failure
Audit integrity	Chain verification result and last check	Verify or export
Access review	Users, roles, last access, exceptions	Revoke or attest
Data flow	Current integrations and data classes	Inspect boundary
Policy versions	Active, pending, retired versions	Compare or approve

11. Component library

Build the following shared components before full page work.

Component	Job	Required props
DecisionCard	State, meaning, next action	state, headline, explanation, actions, freshness
ReasonList	Explain why	reason codes, labels, evidence links
GateStateBanner	Show access state	state, member copy, expiry, safe alternative
WorkQueueRow	Turn a signal into owned work	priority, person, reason, change, due, owner, action
ChangeStatement	Explain trend in words	current, comparison, direction, period, missingness
TrendPanel	Show a time series with meaning	series, range, bands, markers, table fallback
FreshnessLabel	Show age of data	computed time, newest evidence time, stale state
EvidenceDrawer	Show provenance	source references, policy, summary validation
ReviewBadge	Show human-review state	pending, approved, corrected, rejected
ModelDraftPanel	Hold model content safely	draft, citations, validator result, review actions
OwnerChip	Show responsibility	person or team, due state
MetricCard	Aggregate result with denominator	value, numerator, denominator, period, change
EmptyState	Explain why nothing appears	reason, expected next step, retry or support action
ServiceState	Explain loading, partial, stale, and failed states	service, state, fallback, last success

12. Visual tokens

Keep Steady's identity, but split brand colors from semantic text colors.

12.1 Base tokens

Token	Value	Use
surface.canvas	#F7F1E8	Main background
surface.card	#FBF8F2	Cards and panels
surface.subtle	#DCE4D4	Quiet grouped content
text.primary	#2F3A33	Main text
text.secondary	#545E53	Secondary text
border.default	rgba(47,58,51,.16)	Card and table boundary
brand.sage	#A8B8A1	Decorative fill and selected background
action.primary	#2F3A33	Primary button background
action.primaryText	#F7F1E8	Primary button text
focus.ring	#2F3A33	Visible keyboard focus

12.2 Semantic tokens

State	Text or icon	Background	Secondary sign
Safe or complete	#3F6A43	#DCE9DC	Check icon plus label
Information	#375D70	#E2EDF1	Info icon plus label
Caution	#7A5110	#F3E4C8	Triangle icon plus label
Support or stop	#8A4335	#F3DDD8	Stop or phone icon plus label
Pending review	#5B4B78	#ECE7F3	Clock icon plus label
Unknown	#545E53	#ECE9E2	Question icon plus label

Semantic text colors must pass 4.5:1 against their backgrounds. Chart colors need direct labels or distinct line styles. Color may reinforce meaning but cannot carry it alone.

12.3 Type and density

- Use serif display type for page identity, human explanation, and member-facing completion moments.
- Use sans-serif for controls, tables, labels, measures, and dense clinical work.
- Keep member body copy at 16 points or larger on mobile.
- Use a compact desktop density only in clinician, organization, payer, and audit tables.
- Use a 4-point spacing base with 8, 12, 16, 24, 32, and 48 as the working scale.
- Limit general card radius to 16 pixels on desktop and 20 pixels on mobile. Reserve the 24-pixel soft card for member emotional-support moments.

13. Charts and data explanation

Dashboard charts should use position and length for comparison whenever possible. Nielsen Norman Group notes that these encodings are faster to interpret than area and color alone. [NN/g dashboard chart guidance](#).

Required chart contract

Every chart must supply:

- Title written as a question or answer
- Plain-language interpretation
- Time range and timezone
- Current and comparison value
- Scale and unit
- Missing-data behavior
- Direct series labels
- Event annotations when useful
- Accessible table
- Download or source link for authorized users

Default chart choices

Question	Default chart
How has one measure changed?	Line chart with severity bands and event markers
Which work group is largest?	Horizontal bar chart
Where are people in a funnel?	Ordered step bars with counts and conversion
Has a cohort improved?	Distribution or reliable-change bars, not only an average
Is work backing up?	Aging buckets and median time, not a decorative gauge
Did an individual action happen?	Timeline event, not a chart

Avoid donut charts when users must compare close values. Avoid more than three lines in a default view. Avoid dual axes. Avoid red-to-green scales without labels and patterns.

14. Empty, stale, partial, and failed states

Health software cannot treat “no data” as one condition.

State	Example	Required UI behavior
True empty	No sessions yet	Explain what will appear after the first session
Not due	Assessment not scheduled	Show next due date, not zero
Missing	Expected measure not completed	Show missing state and allowed follow-up
Delayed	Integration data behind	Show last successful update and do not infer current state
Partial	Some services loaded	Render safe sections and name what is unavailable
Unauthorized	Role cannot view data	Do not reveal that a hidden record exists
Failed	Projection cannot load	Show retry and safe fallback; log correlation ID privately
Stale decision	Gate outlived its valid window	Re-run gate or limit activity according to policy

The public issue page pattern of turning database failure into a legitimate empty result must not appear in Steady's clinical views. Empty and error are different states.

15. Action and audit behavior

15.1 No optimistic updates for high-impact actions

Approve, correct, override, contact confirmation, safety review, and notification delivery must wait for server confirmation. Low-risk actions such as expanding a drawer or changing a local filter may update immediately.

15.2 Confirmations

Action	Confirmation
Approve cited summary	Standard confirmation with content hash
Correct summary	Show final text, reason, and superseded version
Override pacing rule	High-risk confirmation with scope and expiry
Attempt safety-stop override	Do not render the action
Mark outreach complete	Require channel, result, and timestamp
Export person-level data	Require purpose, scope, and audit entry

15.3 Action response

Every mutation response should return:

- New presentation object
- Audit event ID
- Effective time
- Result state
- Any follow-up work item
- Any failed downstream delivery state

The client should render the returned object. It should not patch individual fields based on what it hoped happened.

16. Suggested endpoints

```

GET /api/v1/member/today
POST /api/v1/member/check-ins
GET /api/v1/member/progress?period=30d
GET /api/v1/member/practices?availability=today

GET /api/v1/clinical/work-items
GET /api/v1/clinical/people
GET /api/v1/clinical/people/:id/overview
GET /api/v1/clinical/people/:id/timeline
GET /api/v1/clinical/people/:id/measures
POST /api/v1/clinical/reviews/:id/approve
POST /api/v1/clinical/reviews/:id/correct
POST /api/v1/clinical/gates/:id/override

GET /api/v1/organization/overview
GET /api/v1/organization/outcomes
GET /api/v1/organization/operations
GET /api/v1/payer/overview
GET /api/v1/payer/methodology
GET /api/v1/audit/events

```

All responses are tenant scoped on the server. The client never supplies a trusted tenant ID or role. Cursor pagination is required for work items, people, timeline, and audit events.

17. Web and iOS rendering boundary

Web

- Server components may load the initial role projection.
- Client components manage filters, drawers, chart range, and action dialogs.
- Route-level error boundaries must distinguish unavailable service, unauthorized access, and true empty results.
- Tables need keyboard navigation, visible focus, row actions, and responsive card fallbacks.

iOS

- Map shared JSON contracts into immutable Swift view models.
- Keep local deterministic safety checks as an offline safety floor.
- Never invent a server delivery or review state while offline.
- Today may show a locally safe fallback plan when disconnected, labeled “On this device.”
- Use Dynamic Type, VoiceOver labels, reduced-motion behavior, and non-drag alternatives for every slider-driven safety input.
- Preserve the session state machine. Redesign its framing and completion summary without merging or skipping safety states.

18. Implementation sequence

Phase 0: Presentation contract

- Add shared `DecisionSurface`, evidence, freshness, action, and review types.
- Add notification delivery states.
- Add server-side role projection services.
- Add fixture builders for every status and failure state.
- Freeze direct raw-event consumption in new UI code.

Exit: member Today and clinician Work Queue can be rendered entirely from fixtures with no client-side domain calculations.

Phase 1: Tokens and components

- Replace low-contrast semantic text tokens.
- Build `DecisionCard`, `GateStateBanner`, `WorkQueueRow`, `FreshnessLabel`, `EvidenceDrawer`, `TrendPanel`, `ReviewBadge`, and service states.
- Add Storybook or an equivalent component gallery on web.
- Add SwiftUI previews for all member states.

Exit: component accessibility tests and screenshot baselines pass.

Phase 2: Member shell

- Build Today, Practice, Progress, Support, and profile navigation.
- Connect check-in result directly to the safe next action.
- Move the catalog out of Today.

- Add plain-language progress summaries and chart tables.
- Correct the notification language state machine.

Exit: a member can complete the main daily path without browsing the catalog or interpreting a raw score.

Phase 3: Clinical cockpit

- Build Work Queue and caseload.
- Build Person Overview, measures, timeline, plan, and review drawer.
- Add ownership, due state, duplicate collapse, freshness, and citation access.
- Implement approve, correct, and allowed override actions with full audit response.

Exit: a clinician can identify the next person, understand why, act, and verify what was recorded from one flow.

Phase 4: Organization, payer, and trust workspaces

- Build stable aggregate metric groups.
- Add numerator, denominator, cohort, time, and missingness to every metric.
- Add role-specific exports.
- Build system health, access review, policy, and audit views.

Exit: each role can answer its first question without entering another role's workspace.

Phase 5: Human-factors validation

- Run role-based usability sessions with fabricated scenarios.
- Measure time to first correct action, error rate, interpretation accuracy, and confidence.
- Run accessibility review with keyboard, screen reader, zoom, Dynamic Type, reduced motion, and color-vision simulation.
- Tune alert thresholds and duplicate suppression from pilot data under governance.

Exit: release criteria below are met with documented evidence.

19. Engineering backlog

Epic	Required output
GUI-01 Presentation contracts	Shared schemas, validators, fixture factory, versioning
GUI-02 Role projections	Member, clinical, organization, payer, audit read models
GUI-03 Design tokens	Accessible semantic palette, type, spacing, focus, chart tokens
GUI-04 Shared components	Decision, gate, queue, trend, evidence, review, state components
GUI-05 Member Today	Check-in-to-action flow, fallback, alternatives, support
GUI-06 Member Progress	Interpretation, trends, markers, table, source history
GUI-07 Clinical Work	Queue groups, ownership, due state, duplicate collapse
GUI-08 Person Record	Overview, measures, timeline, plan, actions, right rail
GUI-09 Review actions	AI draft, validation, approval, correction, allowed override
GUI-10 Aggregate views	Organization and payer metrics with definitions
GUI-11 Trust workspace	System state, audit, access review, data flow, policy versions
GUI-12 Quality	Accessibility, visual regression, permissions, failure states, telemetry

20. Acceptance criteria

20.1 Universal

- Every state card answers state, reason, action, and freshness.
- Every action shown is authorized for that role and current state.
- No client component recalculates safety or priority.
- Empty, stale, partial, failed, and unauthorized states render differently.
- Status never relies on color alone.
- Normal text meets WCAG AA contrast.
- Keyboard focus remains visible and unobscured.
- Charts have accessible tables and direct series labels.
- No analytics event contains free text, measure answers, or person identifiers.

20.2 Member

- Today shows no more than one primary and two secondary actions.
- The check-in result changes the next action without requiring navigation to a catalog.
- Every limited or stopped state gives a safe alternative.
- Support remains reachable during offline, write failure, and service failure states.
- The app never says a care team was notified without a delivered event.
- The member can view and change companion memory controls.

20.3 Clinician and care manager

- Queue order is stable for the same policy version and evidence set.
- Each work item states why it exists and when the evidence arrived.
- Duplicate events collapse without losing count or evidence.
- Approve, correct, and override remain separate actions and audit types.
- Safety-stop override does not appear.
- Person summary claims open their cited evidence.
- Cross-tenant and unauthorized person requests return no record detail.

20.4 Organization and payer

- Every rate includes numerator and denominator.
- Every metric includes time range and cohort definition.
- Missingness and exclusions are visible.
- Small cohorts follow the approved suppression rule.
- Individual drilldown is absent unless explicitly authorized.
- Export content matches the on-screen filters and records them in audit history.

21. Test matrix

Test family	Required cases
Contract	Schema version, unknown state, missing evidence, stale data, action permission
Gate presentation	Open, caution, limited, review, stop, unknown, expiry, offline
Notification truth	Not configured, queued, delivered, acknowledged, failed
Queue	Order, duplicate collapse, owner, due time, reassignment, resolved state
Person record	New event, corrected summary, retired policy, delayed integration
Aggregate	Denominator, missingness, cohort filter, small-cell suppression
Permission	Member, clinician, manager, supervisor, organization, payer, security
Accessibility	Keyboard, VoiceOver, screen reader, zoom, Dynamic Type, reduced motion
Visual regression	All semantic states at mobile, tablet, laptop, and wide desktop
Failure	Projection error, timeout, partial response, stale cache, write conflict
Audit	Action ID, actor, role, evidence, old state, new state, result

22. Product telemetry

Measure whether the GUI reduces work, not only whether it receives clicks.

Member measures

- Time from opening Today to starting the primary action

- Check-in completion to recommended-action start
- Abandonment by screen and state
- Use of safe alternatives
- Support access during failure states
- Progress-interpretation accuracy in usability testing

Clinician measures

- Time to first meaningful action
- Work-item aging by priority
- Duplicate alerts suppressed
- Alert-to-action rate
- Summary approval, correction, and rejection rates
- Evidence-drawer use before high-impact actions
- Queue items transferred or returned because ownership was unclear

Organization measures

- Time to answer a defined program question
- Metric-definition opens
- Export frequency and purpose
- Difference between displayed and exported cohort counts

Telemetry must use coded event types and pseudonymous IDs. It must not record member free text, message content, trigger names, measure answers, or clinical notes.

23. Final engineering rules

- 1 Do not make the client infer domain meaning.
- 2 Do not show an alert without a clear owner and possible action.
- 3 Do not show a percentage without its denominator.
- 4 Do not show a gate without a reason and a safe alternative.
- 5 Do not show model text without its status and evidence.
- 6 Do not claim delivery from an attempted write.
- 7 Do not use the same home screen for every role.
- 8 Do not hide stale or missing data behind a clean chart.
- 9 Do not let visual polish weaken Steady's safety or audit boundaries.
- 10 Make the shortest path through every screen match the user's actual job.

The intended result is not a prettier data system. It is a system that helps a member choose the next safe step, helps a clinician find and complete the next needed review, and helps an organization understand whether care is reaching people and changing outcomes.

Source notes

- [Current Steady public environment](#), inspected August 28, 2026.
- [Current Steady web repository](#), commit c39447a715e92b4800889c7afd6d015e39457c8f.
- [Current Steady iOS repository](#), commit f9a2645fc57135dfe1c7ee61c0e327a4ddc35a0d.
- [NeuroFlow digital behavioral health](#).
- [NeuroFlow solutions](#).
- [NeuroFlow measurement-based care](#).
- [Onward by NeuroFlow on the App Store](#).
- [Blueprint guided measurement-based care](#).
- [Blueprint for Clients on the App Store](#).
- [Greenspace platform](#).
- [Greenspace benchmarks dashboard](#).
- [Osmind mental health EHR](#).
- [Osmind patient portal](#).
- [Osmind Care on the App Store](#).
- [Spring Health Guide](#).
- [Spring Compass provider overview](#).
- [Eleos documentation product](#).
- [AHRQ alert fatigue primer](#).
- [HealthIT.gov clinical decision support](#).
- [W3C WCAG 2.2 reference](#).
- [NN/g dashboard chart guidance](#).

24. Expanded web and analytics coding annex

This annex extends the August 28, 2026 GUI and decision-surface handoff. It converts the full web walkthrough and analytics examples into implementation contracts for engineering.

Complete web atlas	80 proposed screens across patient/member, clinician, organization, payer, review/admin, public and shared access states
Page examples	20 vector mockups showing hierarchy, actions, evidence and failure boundaries
Analytics library	22 role-specific chart screens; eight full chart specifications illustrated in this annex
Clinical presentation	One governed patient projection, stable queue rules, evidence links, action contracts and audit response
Architecture	Event ledger, deterministic policy, scoring, projection services, role APIs, components, permissions and degraded behavior
Delivery	A-to-E integration, phased backlog, test matrix, release gates and acceptance requirements

Non-negotiable boundaries

- Deterministic safety gates remain authoritative. AI cannot make, clear, reverse or override them.
- Safety-relevant answers persist and evaluate immediately. Incomplete assessments never receive a final total.
- Positive screening narrows routing. Negative screening never proves readiness.
- Pilot activity remains stabilization and preparation only. This document does not approve trauma reprocessing or real-person clinical use.
- Corrections append and supersede. They do not erase history.
- Tenant, role, consent, purpose and minimum-necessary checks happen before a presentation object reaches the browser.
- Organizations and payers receive aggregate projections. They do not inherit the clinician person view.

Repository baseline to verify before implementation

Web	tprawlings-lang/Emdr at c39447a715e92b4800889c7afd6d015e39457c8f	Confirm current head, then map old routes to the target screen atlas
Event spine	ADR 0010 ledger, dual-write, backfill and projection parity reported complete; sole event write path pending	Do not build new screens on direct table writes
Identity and tenancy	ADR 0011 person/account split and tenant enforcement reported complete	Every query and projection requires TenantContext
Test baseline	327 unit/safety tests and 17 end-to-end tests reported passing	Retain baseline and add presentation, projection and accessibility coverage

25. Visual system and information grammar

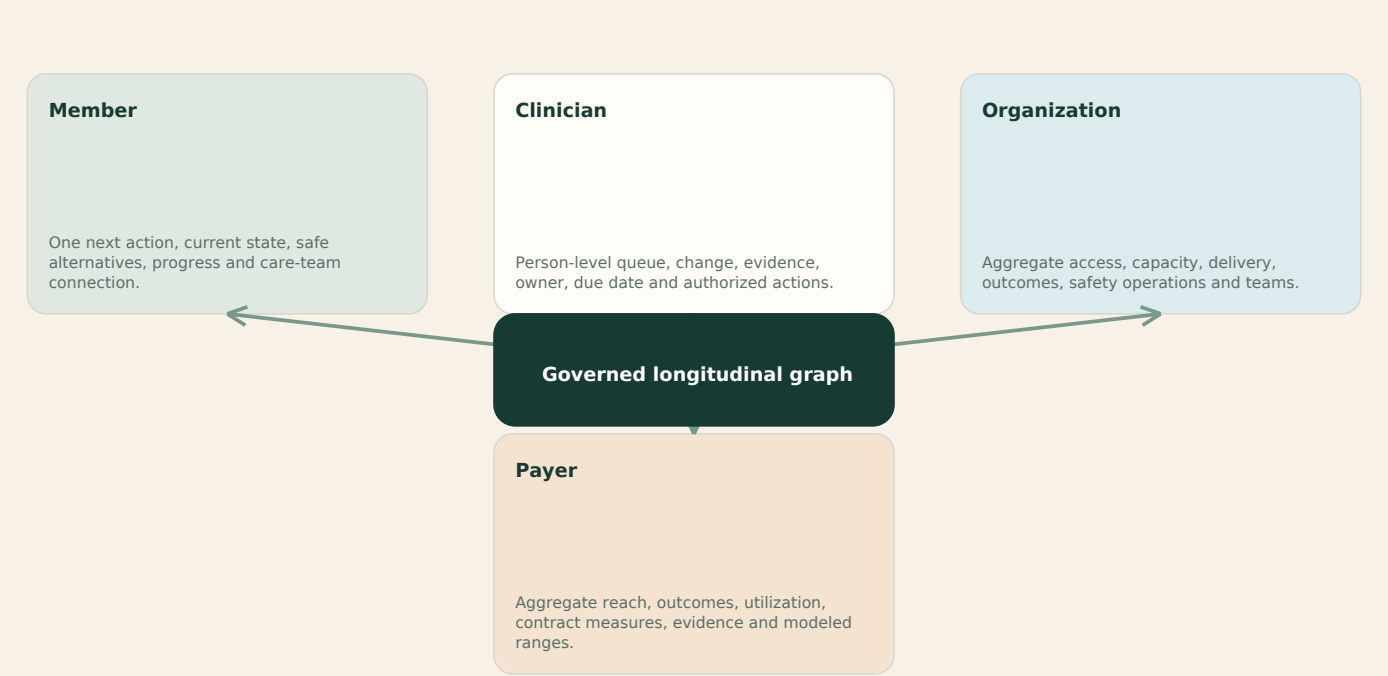
The calm brand remains. The change is information order, density and role authority.

Action	One primary next action and a short accountable queue	Derived from allowedActions in the projection, never inferred in the client
Meaning	What changed, why it matters, freshness, owner and due date	Typed server projection with reason codes and evidence references
Evidence	Sources, measures, answers, rule versions, model status and citations	Drawer or dedicated page; permission checked separately
Raw record	Event replay, audit history and correction chain	Dedicated review route; never the default clinician home

Semantic tokens

ground	Dark green text and primary action	Do not use as a large decorative fill on patient pages
sage	Calm, current, available and measured	Pair with text; never rely on color alone
warm	Review, pending, stale or changed	Must state owner and next action
danger	Fixed block, error or urgent support	Reserved for states requiring immediate attention
info	Method, source, system state and observed result	Use for explanatory boundaries
ivory/linen	Page and card surfaces	Preserve contrast; avoid gray-on-ivory small text

Role separation



Same evidence spine. Different role, authority, granularity and action.

SECTION 26

Complete 80-screen web atlas

Every target screen has a route, user question, visual form and primary action. Engineering may alter route names only through an approved migration map.

DESIGN RULE

Design from the decision backward. Build from the evidence forward.

Patient and member - 15 screens

15 screens. Each screen must implement loading, ready, empty, stale, partial, permission-denied and failed states when those states can occur.

	URL	Primary purpose	Secondary purpose / state	Primary action
Welcome	/app/welcome	Understand scope and control	Orientation flow	Continue
Consent and sharing	/app/consent	Choose storage and care-team sharing	Consent projection	Save choices
Baseline screening	/app/screening	Create a starting point	Instrument form and progress	Next question
Today	/app/today	Know the safest next step	One primary action, two secondary	Start check-in
Daily check-in	/app/check-in	Report current state	Low-burden questions	Continue or pause
Activities	/app/activities	Choose an allowed support tool	Approved activity list	Open activity
Session preparation	/app/session/prepare	Confirm readiness and environment	Readiness and fixed gates	Begin check
Active session	/app/session/:id	Follow the current activity safely	Pause, stop, grounding, support	Pause or continue
Safety gate	/app/session/:id/safety	Reach support and understand the stop	Fixed rule, reason, alternatives	Get support
Session close	/app/session/:id/close	Return to baseline and plan follow-up	Grounding and close state	Complete close
Progress	/app/progress	See patterns with context	Measures, state, activities	Open evidence
Care plan	/app/plan	Know what is active and why	Versioned plan and authority	Message care team
Messages	/app/messages	Communicate securely	Care-team thread and boundary	Send
Care team	/app/care-team	See verified access	Provider role and consent	Message or revoke
Profile and privacy	/app/settings	Manage account and data choices	Identity, sharing, requests	Save changes

Role-level acceptance

- No page presents more than one dominant action.
- Pause, stop, grounding and support remain reachable where session activity appears.
- No score is presented as diagnosis, readiness or proof of improvement.

Clinician - 14 screens

14 screens. Each screen must implement loading, ready, empty, stale, partial, permission-denied and failed states when those states can occur.

Screen	URL	Core question	Visual form / state	Primary action
Today	/clinician/today	Know who needs attention	Priority queue	Review now
Caseload	/clinician/caseload	Scan change, owner and freshness	Stable ordered rows	Open person
Patient overview	/clinician/member/:id	Understand current state	Change, meaning, action, evidence	Start review
Safety review	/clinician/member/:id/safety	Respond to fixed gate event	Rule result and response log	Document response
Measures	/clinician/member/:id/measures	Review validated change	Instrument-specific chart	Open answers
Sessions	/clinician/member/:id/sessions	Review session pattern	Concise list and gate state	Open session
Session detail	/clinician/member/:id/session/:sid	Understand state and response	Human-readable event sequence	Sign or correct
Care plan	/clinician/member/:id/plan	Review active plan and versions	Authority and next review	Draft change
Messages	/clinician/messages	Communicate with context	Secure inbox	Reply
Referrals	/clinician/referrals	Move intake to first contact	Eligibility, consent, wait, owner	Contact or assign
Handoffs	/clinician/handoffs	Keep accountability through transfer	From, to, reason, due, state	Accept
Schedule	/clinician/schedule	See visits and reviews	Calendar plus overdue actions	Open item
Reports	/clinician/reports	Review team quality	Aggregate clinical operations	Open report
Patient audit	/clinician/member/:id/audit	Trace access and decisions	Immutable filtered events	Open evidence

Role-level acceptance

- Queue order is stable and explainable.
- Every alert has evidence arrival time, owner and possible action.
- Person claims open cited evidence without a second search.

Organization - 9 screens

9 screens. Each screen must implement loading, ready, empty, stale, partial, permission-denied and failed states when those states can occur.

Screen	Endpoint route	Key questions	Visual forms / data	Primary action
Operating overview	/organization/overview	See network change and action	Aggregate summary and queue	Review location
Access pipeline	/organization/access	Find referral loss and wait	Funnel and wait trend	Open action list
Capacity	/organization/capacity	Balance demand and slots	Location comparison	Rebalance
Care delivery	/organization/care-delivery	Track continuity and handoffs	Completion and review coverage	Open workflow
Outcomes	/organization/outcomes	See observed cohort patterns	Coverage, change, missingness	Open cohort
Safety operations	/organization/safety	Monitor response workflow	Gate volume and response time	Open action
Teams	/organization/teams	Compare workload and overdue work	Team-level operations	Open team
Locations	/organization/locations	Compare sites with context	Volume, wait, access, capacity	Open location
Reports	/organization/reports	Create review-ready packets	Governed exports	Download

Role-level acceptance

- Every percentage includes numerator and denominator.
- Cohort, range, refresh, exclusions and missingness are visible.
- No person-level drilldown exists unless separately authorized.

Payer - 10 screens

10 screens. Each screen must implement loading, ready, empty, stale, partial, permission-denied and failed states when those states can occur.

Screen	Endpoint route	User question	Visual items / data	Primary action
Population overview	/payer/overview	See reach, outcome and utilization	Observed summary	Open methods
Engagement	/payer/engagement	See eligibility to active use	Population funnel	Open cohort
Outcomes	/payer/outcomes	Review observed cohort change	Coverage and missingness	Open rules
Utilization	/payer/utilization	Review claims-based use	Per-1,000 trend and lag	Open claims method
Access	/payer/access	Review time to care	Wait, start, abandonment	Open action
Cohorts	/payer/cohorts	Reproduce contract populations	Versioned definitions	Open cohort
Population access	/payer/population-access	Find access gaps	Approved groups and suppression	Open method
Evidence	/payer/evidence	Separate observed and modeled	Metric and model registry	Open evidence
Contract reports	/payer/contract	Review agreed measures	Observed versus target	Download
Data quality	/payer/data-quality	Understand coverage and lag	Feed quality and exclusions	Open audit

Role-level acceptance

- Every percentage includes numerator and denominator.
- Cohort, range, refresh, exclusions and missingness are visible.
- No person-level drilldown exists unless separately authorized.

Review and administration - 13 screens

13 screens. Each screen must implement loading, ready, empty, stale, partial, permission-denied and failed states when those states can occur.

	URL	Primary action	Secondary action	Primary screen
Review home	/review	See open decisions and release gates	Review queue	Open gate
Access requests	/review/access	Approve scoped access	Role, purpose, expiration	Approve or deny
Clinical review	/review/clinical	Review language and flow	Version, evidence, decision	Record review
Autonomous flow	/review/autonomous	Inspect bounded AI path	Inputs, policy, module, draft, review	Approve or correct
BLS oversight	/review/bls	Review pacing and stop controls	Approved configurations	Record review
Testing console	/review/testing	Judge release behavior	Suites by visible outcome	Inspect failure
Safety rule results	/review/safety	Replay fixed scenarios	Expected, actual, resources	Open trace
Audit trail	/review/audit	Trace all governed events	Immutable filters	Open event
Data lineage	/review/lineage	Trace screen statement to source	Event to projection chain	Replay
Research workspace	/review/research	Use approved de-identified data	Consent and cohort guardrails	Request export
Release gates	/review/release	Record required sign-offs	Owner, evidence, state	Approve or block
Demo data	/review/demo-data	Reset and verify fabricated data	Seed, scan, reset	Run identity scan
System status	/review/status	See health and safe fallback	Service, version, degraded mode	Inspect

Role-level acceptance

- Every decision records actor, role, version, evidence and time.
- High-impact actions are confirmed and not optimistically shown as complete.
- Release gates cannot be bypassed from ordinary admin controls.

Public institutional site - 11 screens

11 screens. Each screen must implement loading, ready, empty, stale, partial, permission-denied and failed states when those states can occur.

Screen	Platform route	Key questions	Visual design / state	Primary action
Home	/	Choose the relevant platform path	Status, three products, evidence	Request review
Platform	/platform	Understand one system and three surfaces	Working product sequence	Explore
Steady Personal	/personal	Review member value and limits	Member page examples	Review demo
Steady Clinical	/clinical	Review care-team workflow	Queue and person examples	Review demo
Steady Intelligence	/intelligence	Review aggregate intelligence	Organization and payer examples	Explore
Organizations	/organizations	Review operating value	Access, capacity, action queue	Request review
Payers	/payers	Review evidence and contract value	Observed and modeled examples	Request review
Trust	/trust	Review safety, privacy and security	Control map and gaps	Open control
Evidence	/evidence	Separate demonstrated, validating and target	Claim registry	Open source
FAQ	/faq	Resolve common objections	Role-grouped answers	Request review
Request review	/request-review	Request scoped access	Role and purpose form	Submit

Role-level acceptance

- The current demonstration boundary remains visible.
- Denied and missing pages do not reveal protected existence.
- Keyboard and screen-reader paths reach every action.

Shared access states - 8 screens

8 screens. Each screen must implement loading, ready, empty, stale, partial, permission-denied and failed states when those states can occur.

Screen	Endpoint route	User question	Visual forms / state	Primary action
Sign in	/login	Enter secure workspace	Organization or invitation access	Sign in
Verification	/verify	Complete second factor	Six-digit code	Verify
Password reset	/reset	Recover account	Approved email flow	Send link
Accept invitation	/invite/:token	Join scoped review	Role, purpose, expiration	Accept
No access	/403	Understand denied scope	Reason and request path	Request access
Session expired	/session-expired	Re-enter safely	No retained unsaved text	Sign in
Degraded service	/status/degraded	Use safe fallback	Available and blocked functions	Open grounding
Not found	/404	Return to an allowed path	No data leak	Return home

Role-level acceptance

- The current demonstration boundary remains visible.
- Denied and missing pages do not reveal protected existence.
- Keyboard and screen-reader paths reach every action.

SECTION 27

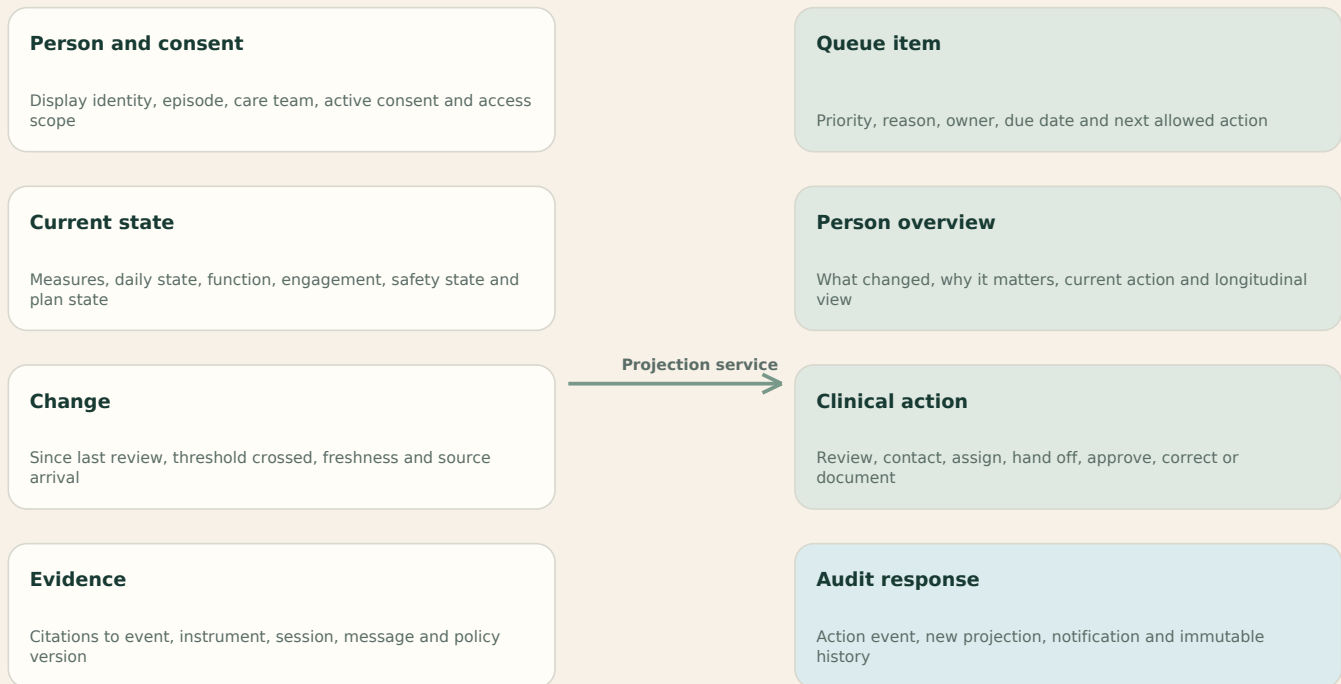
Patient-to-clinician presentation model

The clinician never receives a loose pile of patient events. A governed projection turns longitudinal evidence into a current state, change, reason, owner, action and traceable evidence bundle.

DESIGN RULE

Design from the decision backward. Build from the evidence forward.

27.1 End-to-end presentation architecture



Patient data never becomes a raw table on the clinician home screen.

The projection is a read model, not a new source of truth. Actions append domain events. Projection workers rebuild the clinician queue and patient summary from those events.

27.2 Clinician patient projection contract

```
ClinicianPatientProjection {
  schemaVersion, projectionVersion, generatedAt
  tenantId, personId, careEpisodeId
  identity { displayId, displayName, pronouns, careTeam }
  access { consentState, roleScope, purpose, expiresAt }
  currentState { measures, dailyState, function, engagement,
  safetyState, carePlanState }
  change { sinceLastReview, reasonCodes, crossedRules,
  evidenceArrivedAt, freshness }
  priority { level, sortKey, reason, duplicateGroupId }
  accountability { ownerId, ownerRole, dueAt, handoffState }
  allowedActions[]
  forbiddenActions[]
  evidenceRefs[]
  modelContent { status, modelVersion, evidenceRefs, reviewedBy }
}
```

Field Name	Required Behavior
identity	Use minimum-necessary display identity. Legal identity stays out of routine views unless required.
access	Projection generation stops if tenant, role, consent, purpose or expiration fails.
currentState	Each domain keeps source, valid range, completion state and freshness.
change	Computed against explicit last-review or baseline anchor. Never compare to an unknown date.
priority	Server-owned sort key. Client must not reconstruct priority.
allowedActions	Server-authorized actions with confirmation and audit requirements.
evidenceRefs	Opaque references resolved through a separately authorized evidence endpoint.
modelContent	Draft, reviewed, approved, corrected or rejected. Model text is never shown without status.

27.3 Stable clinician work-queue ordering

	Event type	Event description	Event context	Event action
1	Fixed safety event	Open pause or block with response due	Rule result, time, support shown, owner	Review and document
2	Overdue accountable work	DueAt before now and unresolved	Reason, due time, owner, handoff	Complete or hand off
3	Meaningful validated change	Instrument or approved rule crossed	Change, range, baseline, source time	Clinical review
4	Waiting/access gap	Referral or message beyond policy window	Time waiting, last attempt, owner	Contact or assign
5	Due care work	Plan, note, measure or review due	Due date and required evidence	Open work
6	Informational	No action required	Short context only	Open if needed

Duplicate collapse

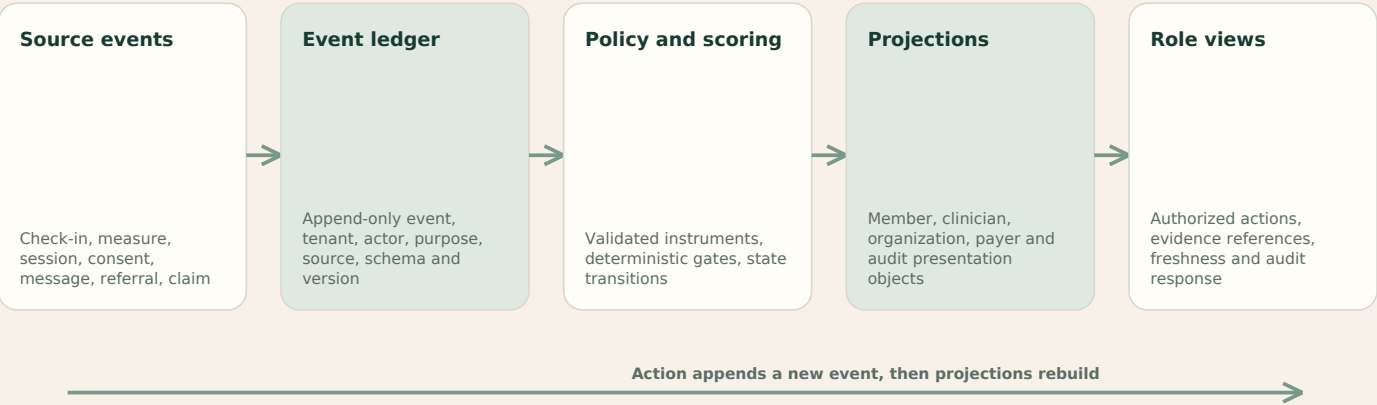
- Events with the same person, decision and due window collapse into one queue item.
- The queue item retains every evidence reference and states how many events were combined.
- A newer fixed safety event never collapses behind a lower-priority measure change.
- A correction updates the projection and appends history. It does not delete the earlier evidence.

27.4 Person overview information order

Order	Section	Content	State
1	Identity and access header	Display identity, episode, care team, consent state, owner	No hidden access assumptions
2	What changed	Since-last-review measures, state, session, engagement and safety events	Maximum three high-signal items
3	Why it matters	Typed reason, rule, comparison window and uncertainty	No unsupported narrative
4	Next action	Allowed action, owner, due date and confirmation requirement	One primary action
5	Progress view	Instrument small multiples, session response, function and goals	Separate scales and visible missingness
6	Unified timeline	Events grouped into human-readable episodes	Raw event IDs remain in evidence
7	Evidence drawer	Sources, versions, model status, audit and corrections	Permission checked on open

Write path

Read path



27.5 Clinician actions and audit response

Action	Precondition	Postcondition	Event Name
Acknowledge	Action is informational or assigned	Queue state updates after event commit	work_acknowledged
Review	Evidence and scope remain current	Review record and projection rebuild	clinical_review_recorded
Assign	Target has valid tenant role	Owner and due date update	work_assigned
Handoff	Receiving user accepts or policy permits pending	Handoff state changes	handoff_created / accepted
Correct	Source or summary error documented	New event supersedes projection value	correction_appended
Approve model text	Reviewer role and cited evidence valid	Status changes to approved	model_content_approved
Document safety response	Fixed event exists and user is authorized	Response attached; gate history unchanged	safety_response_documented

Safety stops have no ordinary override action. Re-entry is a new fixed-rule evaluation, not a clinician button that clears history.

SECTION 28

Page examples and implementation notes

Twenty representative pages show the intended information hierarchy. These are product specifications, not decorative concepts.

DESIGN RULE

Design from the decision backward. Build from the evidence forward.

Member Today

Patient or member | Target route: /app/today

Steady

Patient or member

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Member Today

Action first. Meaning second. Evidence third.

Start check-in

Choose the safest next step. Start check-in. Your answer selects the safest available next step.

Recent pattern

Distress moved from 6 to 4. Pattern only, not a diagnosis.

60-second reset

Grounding available now.

Learn

Understanding triggers. 4 minutes.

Care team

Last reply yesterday.

Start check-in

Choose the safest next step	member_today.v4	Start check-in; open grounding; message care team	loading, ready, gate, offline	One primary action; sources and limitations stay visible

STEADY WEB GUI, ANALYTICS AND CLINICAL PRESENTATION

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Daily Check-in

Patient or member | Target route: /app/check-in

Steady

Patient or member

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Daily Check-in

Action first. Meaning second. Evidence third.

Daily check-in - question 2 of 4

0 calm

2

4

6

8 intense

This answer can change which activities are available today.

Continue

Pause

How activated do you feel right now?

Report current state with low burden	member_checkin.v5	answer; pause; open support	partial, safety-positive, saved, failed	Safety-relevant answers persist and evaluate immediately

Deterministic Safety Gate

Patient or member | Target route: /app/session/:id/safety

Steady

Patient or member

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Deterministic Safety Gate

Action first. Meaning second. Evidence third.

SAFETY CHECK - SESSION PAUSED

Let's make sure you have support right now.

A direct answer met fixed rule S-04. No AI model made or cleared this decision.

Why this appeared

Fixed safety answer and policy version are available in evidence.

Call or text 988

Contact my care team

I am safe now - check again

Reach support and understand why activity stopped	gate_view.v7	call; contact; recheck	pause, block, re-entry, service failure	No AI authority; support remains reachable on failure

Member Progress

Patient or member | Target route: /app/progress

Steady

Patient or member

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Member Progress

Action first. Meaning second. Evidence third.

Check-ins

18
4 this week

Activities

12
7 grounding

Current trend

Improving
PHQ-9 down 4

Symptoms and daily distress

Month	Symptoms and daily distress (approximate value)
Mar	3.5
Apr	3.2
May	2.8
Jun	2.5
Jul	2.2
Aug	2.8

90-day pattern with comparison window and sources.

Read this first

Recent rise crosses review rule.
Open context before changing the plan.

Clinician Work Queue

Steady Clinical | Target route: /clinician/today

Steady

Steady Clinical

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Clinician Work Queue

Action first. Meaning second. Evidence third.

1 safety review needs attention now

Review now

Member	What changed	Freshness	Owner	Action
Demo A-142	Safety rule met	23 min	You	Review
Demo B-317	PHQ-9 up 5	Today	J. Lee	Open
Demo C-082	No first contact	2 days	Unassigned	Assign
Demo D-204	Distress 7 to 3	Yesterday	You	Sign note

Know who needs attention and why	clinician_queue.v8	review; assign; contact; sign	empty, stale, failed, safety	Stable order: safety, overdue, change, waiting, due

Clinician Patient Overview

Steady Clinical | Target route: /clinician/member/:id

Steady

Steady Clinical

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Clinician Patient Overview

Action first. Meaning second. Evidence third.

What changed: PHQ-9 increased 5 points in 14 days. Review due today.

Current measure

PHQ-9 14

Safety state

Gate clear

Next action

Clinical review

90-day progress

Mar

Apr

May

Jun

Jul

Aug

Validated measures, daily state and care events use separate scales.

Accountability

Owner: Dr. Rivera

Due: today at 5 PM

Evidence: current

Understand current state and next clinical action	clinician_patient.v12	review; message; handoff; document	current, stale, partial, no consent	Every claim opens cited evidence

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Clinician Measure History

Steady Clinical | Target route: /clinician/member/:id/measures

Steady

Steady Clinical

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Clinician Measure History

Action first. Meaning second. Evidence third.

What changed: PHQ-9 increased 5 points in 14 days. Review due today.

Current measure

PHQ-9 14

Safety state

Gate clear

Next action

Clinical review

90-day progress

Mar

Apr

May

Jun

Jul

Aug

Validated measures, daily state and care events use separate scales.

Accountability

Owner: Dr. Rivera

Due: today at 5 PM

Evidence: current

Review validated score change	measure_history.v6	open answers; compare period; document	missing, incomplete, late, corrected	Instrument scale, range and completion status required

STEADY WEB GUI, ANALYTICS AND CLINICAL PRESENTATION

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Session Response

Steady Clinical | Target route: /clinician/member/:id/session/:sid

Steady

Steady Clinical

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Session Response

Action first. Meaning second. Evidence third.

Activation before and after each session

Aug 1

Aug 6

Aug 11

Aug 16

Aug 22

Aug 27

One session does not prove outcome change.

Clinical reading

4 of 6 closed lower.
1 unchanged.
1 paused by fixed gate.

Next: review paused session.

Review before and close state without overclaiming	session_summary.v5	open timeline; sign; correct	complete, paused, missing close, corrected	One session cannot be treated as clinical outcome proof

Clinician Safety Review

Steady Clinical | Target route: /clinician/member/:id/safety

Steady

Steady Clinical

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Clinician Safety Review

Action first. Meaning second. Evidence third.

Rule S-04 met 23 minutes ago. Session remains blocked.

Gate record

1. Direct response persisted

2. Consent and scope validated

3. Fixed rule evaluated

4. Support options shown

5. Clinician owner notified

Document response

Response note

Contacted member. Confirmed current support

Document response

the gate.

Respond to a fixed gate event	safety_review.v7	contact; document; handoff	open, responded, re-entry, unresolved	Response does not erase or override gate history

Evidence Drawer

Steady Clinical | Target route: shared drawer

Steady

Steady Clinical

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Evidence Drawer

Action first. Meaning second. Evidence third.

Evidence drawer

Claim

PHQ-9 increased 5 points in 14 days

Source

Validated self-report completed today

Rule

measure_change.v6

Safety

S-04.v7 evaluated separately

Model text

Draft summary - review required

Projection

clinician_patient.v12

Audit

Opened by Dr. Rivera at 9:42 AM

Trace a statement to source and processing	evidence_bundle.v4	open source; replay; correct	available, missing, stale, superseded	Shows rule, model, projection and audit versions

Organization Overview

Steady Intelligence | Target route: /organization/overview

Steady

Steady Intelligence

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Organization Overview

Action first. Meaning second. Evidence third.

First contact

2.6 days
0.8 slower

Engaged

72%
3,470 / 4,820

Measure coverage

84%
4,049 / 4,820

Access pathway

Referred

420

Contacted

344

Scheduled

265

Started

236

Referral to contact to scheduled to started.

What changed

North contact slowed after referral volume rose 18%.

Owner: Operations.

Due: Monday.

See network change and accountable action	org_overview.v6	open location; export; assign	small cell, stale feed, partial	Aggregate only; denominator and refresh required

STEADY WEB GUI, ANALYTICS AND CLINICAL PRESENTATION

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Organization Access and Capacity

Steady Intelligence | Target route: /organization/capacity

Steady

Steady Intelligence

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Organization Access and Capacity

Action first. Meaning second. Evidence third.

First contact

2.6 days
0.8 slower

Engaged

72%
3,470 / 4,820

Measure coverage

84%
4,049 / 4,820

Demand versus available slots

North demand

118

North slots

62

Central slots

88

Telehealth slots

152

Next 14 days by location.

Action queue

North has 56 fewer slots than expected referrals.

Owner: Network operations.
Due: today.

Balance demand and available care	org_capacity.v4	rebalance; open location	forecast missing, schedule stale	Forecast and observed data labeled separately

STEADY WEB GUI, ANALYTICS AND CLINICAL PRESENTATION

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Organization Outcomes

Steady Intelligence | Target route: /organization/outcomes

Steady

Steady Intelligence

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Organization Outcomes

Action first. Meaning second. Evidence third.

First contact

2.6 days
0.8 slower

Engaged

72%
3,470 / 4,820

Measure coverage

84%
4,049 / 4,820

Outcome status by month

Improved

300

Stable

155

Worsened

68

Missing

72

Improved, stable, worsened and missing follow-up.

Boundary

Observed cohort pattern.
No causal claim.

Missing follow-up remains in
denominator.

Review observed cohort patterns	org_outcomes.v5	open cohort; export methods	missing follow-up, suppressed	Missingness stays in denominator; no causal claim

STEADY WEB GUI, ANALYTICS AND CLINICAL PRESENTATION

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Payer Population Overview

Steady Intelligence | Target route: /payer/overview

Steady

Steady Intelligence

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Payer Population Overview

Action first. Meaning second. Evidence third.

Engaged

68%

8,486 / 12,480

Follow-up

74%

9,235 / 12,480

Claims lag

60 days

Current

Contract signals

Month	Top Line (%)	Bottom Line (%)
Jan	40	25
Feb	45	28
Mar	48	30
Apr	50	32
May	53	35
Jun	55	40

Reach, follow-up and utilization use separate scales.

Interpretation

Observed results separate from modeled estimates.

Cohort and data lag stay attached.

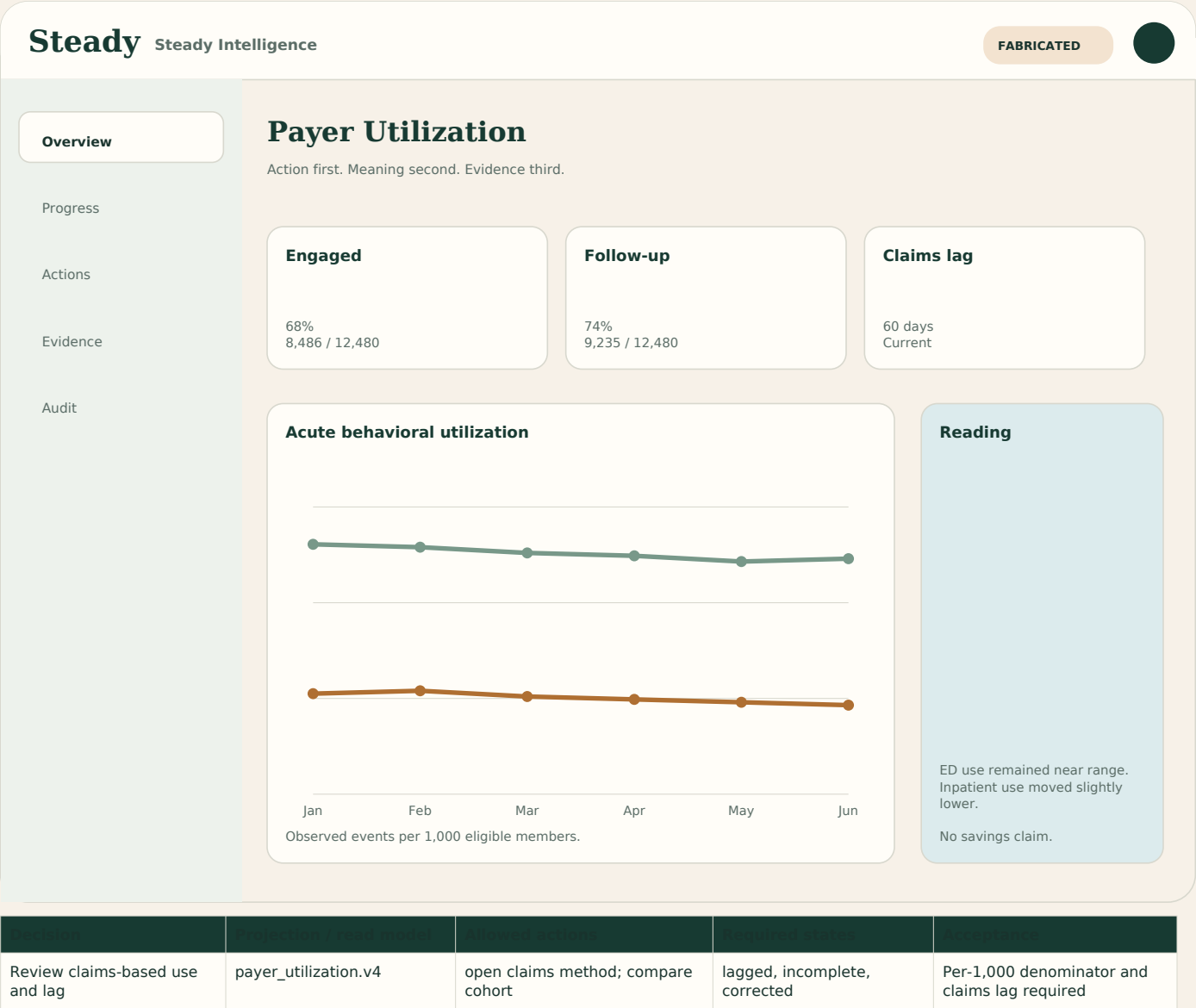
Review reach, outcomes and use	payer_overview.v5	open method; open cohort	claims lag, partial, suppressed	Observed and modeled results are separate

STEADY WEB GUI, ANALYTICS AND CLINICAL PRESENTATION

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Payer Utilization

Steady Intelligence | Target route: /payer/utilization



Payer Cost Model

Steady Intelligence | Target route: /payer/evidence/cost

Steady

Steady Intelligence

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Payer Cost Model

Action first. Meaning second. Evidence third.

Engaged

68%

8,486 / 12,480

Follow-up

74%

9,235 / 12,480

Claims lag

60 days

Current

Estimated cost range

Conservative

\$1-\$7

Expected

\$5-\$13

Upper

\$8-\$20

Modeled PMPM. Not observed savings.

Model boundary

Assumptions and uncertainty required.

Observed claims replace assumptions over time.

Review estimated range and assumptions	cost_model_view.v2	open assumptions; approve model version	draft, approved, superseded	Never label estimated value as observed savings

STEADY WEB GUI, ANALYTICS AND CLINICAL PRESENTATION

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Testing Console

Steady Review | Target route: /review/testing

Steady

Steady Review

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Testing Console

Action first. Meaning second. Evidence third.

Release candidate RC-28

Safety gates	248 / 248	PASS
Consent boundaries	96 / 96	PASS
Care projections	131 / 132	REVIEW
Accessibility	74 / 74	PASS

Scenarios grouped by user-visible decision and gate result.

Judge release behavior	test_summary.v6	inspect failure; assign; block release	running, pass, fail, flaky	Group tests by user-visible outcome

Audit and Lineage

Steady Review | Target route: /review/lineage

Steady

Steady Review

FABRICATED

Overview

Progress

Actions

Evidence

Audit

Audit and Lineage

Action first. Meaning second. Evidence third.

Immutable audit and lineage

9:19	Member	Check-in submitted	event.v3
9:19	Policy	Safety evaluated	S-04.v7
9:20	Projection	Care summary updated	v12
9:42	Clinician	Record opened	RBAC.v5
9:48	Clinician	Review documented	action.v4

Corrections append and supersede. They never erase history.

Trace screen statement to source	lineage_trace.v4	replay; inspect; correct	complete, gap, superseded	Immutable events; corrections append

Institutional Home

Public website | Target route: /

Steady

Public website

FABRICATED

Institutional Home

Action first. Meaning second. Evidence third.

Behavioral health support that continues between visits.

One governed system. Three role-specific products.

Request review

View evidence

Steady Personal

Member support and clear choices

Steady Clinical

Prioritized work and context

Steady Intelligence

Aggregate operations and evidence

Controlled demonstration with fabricated data only. Not clinical or emergency care.

Choose a role path and understand current status	public_home.v3	request review; view evidence	status change, unavailable demo	Fabricated data and current-stage boundary are prominent

STEADY WEB GUI, ANALYTICS AND CLINICAL PRESENTATION

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Trust Center

Public website | Target route: /trust

Steady

Public website

FABRICATED

Trust Center

Action first. Meaning second. Evidence third.

Trust is product behavior, not a policy page

Access control

Tenant, role, consent, purpose and minimum necessary

Safety authority

Fixed gates cannot be relaxed by AI or ordinary override

Auditability

Every access, decision, correction and export is versioned

Failure behavior

Grounding and crisis resources remain available on service failure

Review product controls and known gaps	trust_center.v4	open control; request security review	implemented, validating, target	Trust statements map to product behavior and evidence

SECTION 29

Role-specific analytics and chart contracts

Charts are decision tools. Clinicians see person-level progress. Organizations see aggregate operations and outcomes. Payers see population reach, utilization, contract evidence and separate modeled ranges.

DESIGN RULE

Design from the decision backward. Build from the evidence forward.

29.1 Rules for every chart

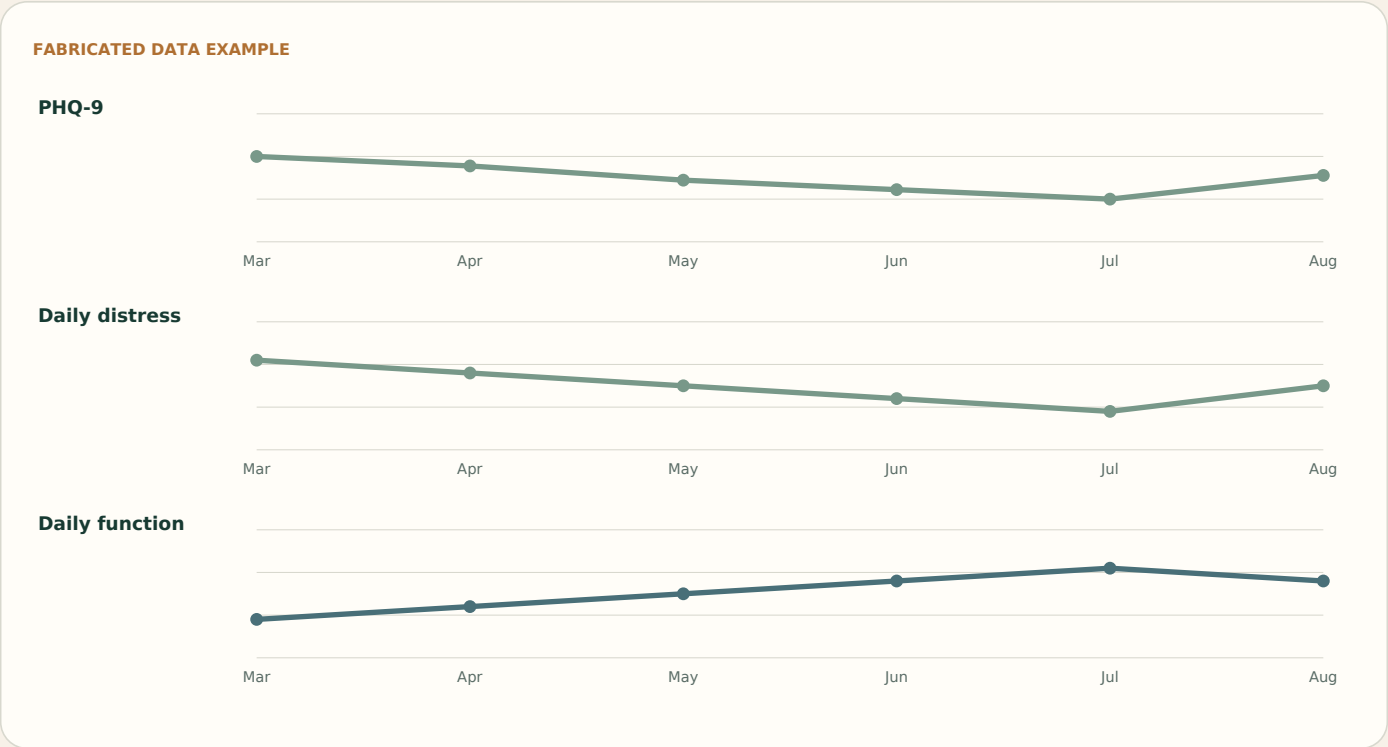
Guideline	Guiding rule
Denominator	Never show a percentage without numerator and denominator in the same view.
Range	Always show time window, baseline or comparison anchor and refresh time.
Missing data	Missing, incomplete, late, rejected and suppressed data remain visible.
Scale	Do not overlay different clinical scales. Use aligned small multiples.
Events	Annotations mark sessions, plan versions or care events. They do not imply cause.
Observed versus modeled	Use separate surfaces, labels and data contracts.
Safety	Show fixed event timelines and response workflow. Do not create a predictive risk score.
Accessibility	Keyboard series controls, direct labels, non-color encoding and screen-reader summary required.
Export	Export must match current filters and write an audit event.

Complete chart-screen inventory

Screen	Content	Count
Clinician	Progress overview; measures; session response; function and goals; engagement; safety timeline; plan response	7
Organization	Operating overview; access; capacity; engagement; outcomes; safety operations; location comparison	7
Payer	Population overview; reach; outcome cohorts; utilization; access; cost model; contract performance; data quality	8

Clinician progress overview

Grain: person-level | Window: 90 days | Data: PHQ-9, daily distress, function



Input	Input metadata	Display rules	Output
Separate small multiples	cohort or person; numerator; denominator; range; refresh; missing; version	No mixed-scale overlay; show freshness and events	typed series, units, bounds, annotations, evidenceRefs

Clinician session response

Grain: person-level | Window: last 6 sessions | Data: pre and close activation

FABRICATED DATA EXAMPLE

Activation before and after each session

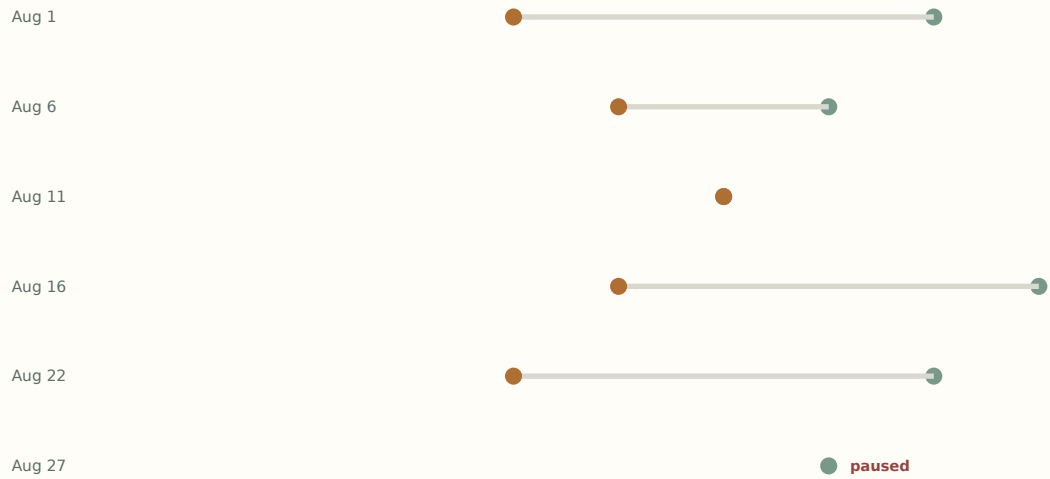


Chart	Input metadata	Display rules	Output
Slope chart	cohort or person; numerator; denominator; range; refresh; missing; version	Missing close remains visible; no treatment claim	typed series, units, bounds, annotations, evidenceRefs

Clinician safety timeline

Grain: person-level | Window: 90 days | Data: fixed gate and response events

FABRICATED DATA EXAMPLE

Fixed gate events and human response



No predictive risk score. Fixed rule, result, support and response remain visible.

Input	Input metadata	Decision rule	API output
Event timeline	cohort or person; numerator; denominator; range; refresh; missing; version	No predictive risk score; policy version required	typed series, units, bounds, annotations, evidenceRefs

Organization access pathway

Grain: aggregate | Window: 30 days | Data: referral stages

FABRICATED DATA EXAMPLE

Referral to care start



Chart	Required metadata	Decision rule	API output
Horizontal funnel	cohort or person; numerator; denominator; range; refresh; missing; version	Count and denominator on every stage	typed series, units, bounds, annotations, evidenceRefs

Organization capacity

Grain: aggregate | Window: next 14 days | Data: demand and open slots

FABRICATED DATA EXAMPLE

Demand compared with open first-visit slots

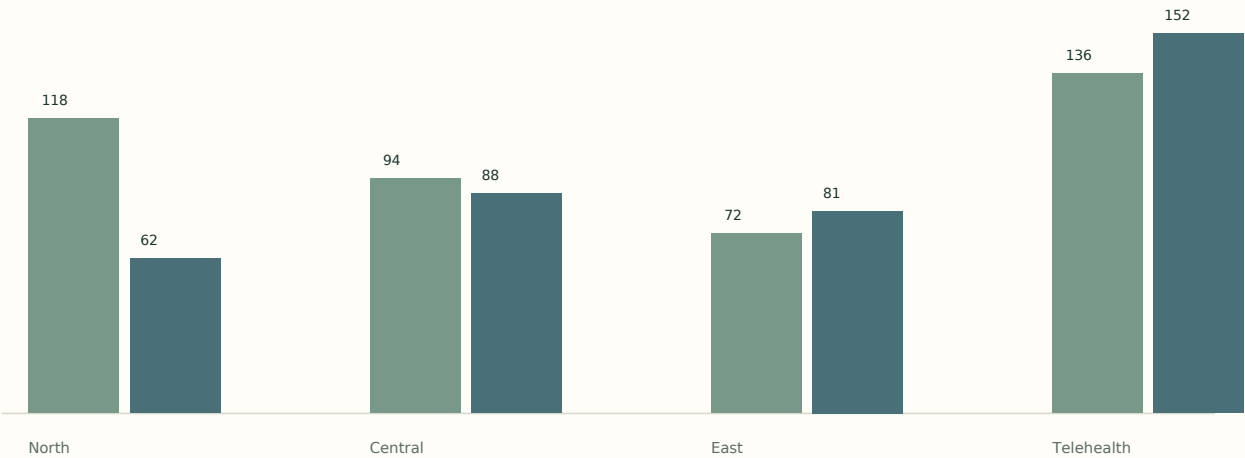


Chart	Required metadata	Decision rule	API output
Grouped bars	cohort or person; numerator; denominator; range; refresh; missing; version	Forecast label and owner required	typed series, units, bounds, annotations, evidenceRefs

Organization outcomes

Grain: aggregate | Window: 6 months | Data: improved, stable, worsened, missing

FABRICATED DATA EXAMPLE

Outcome status with missing follow-up visible



Denominator 595. Missing remains in the total.

type	input metadata	output data	output
Stacked allocation	cohort or person; numerator; denominator; range; refresh; missing; version	Missing follow-up stays in total	typed series, units, bounds, annotations, evidenceRefs

Payer utilization

Grain: aggregate | Window: 6 months | Data: ED and inpatient per 1,000

FABRICATED DATA EXAMPLE

Observed acute behavioral utilization

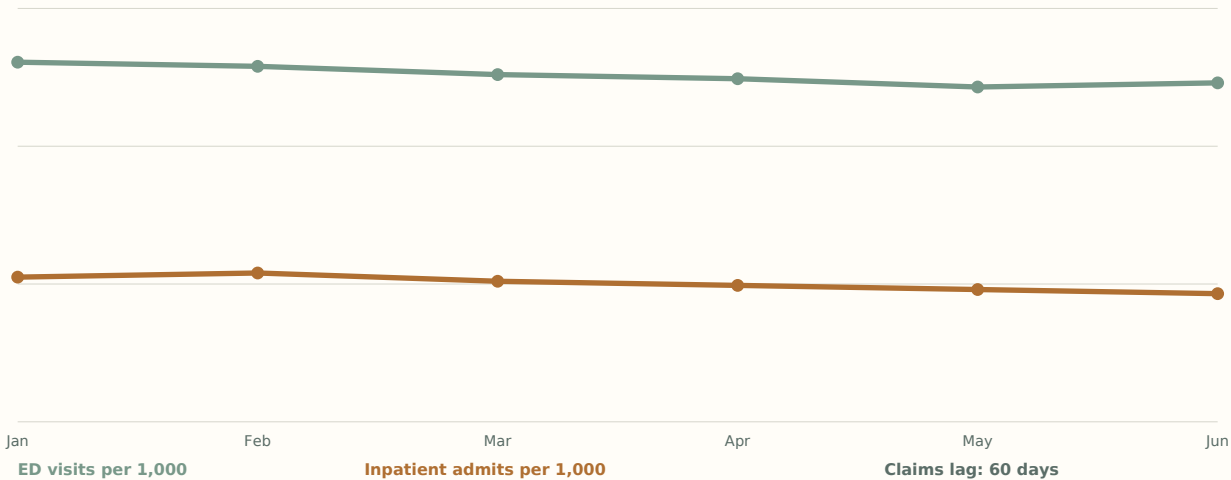


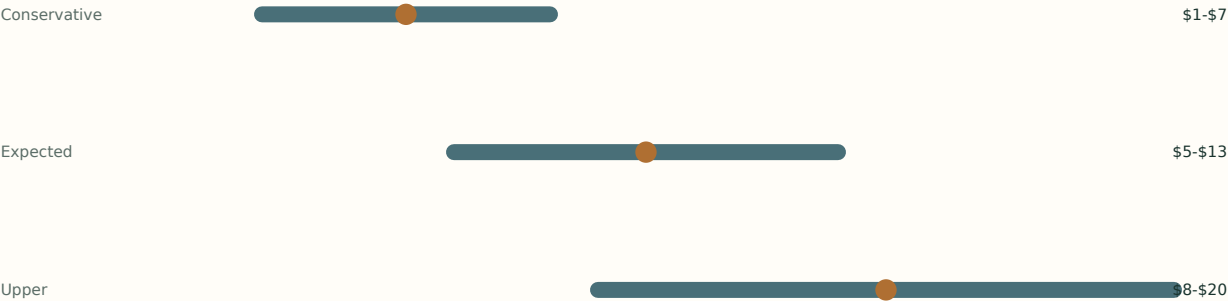
Chart	Required metadata	Required data	API output
Two-series line	cohort or person; numerator; denominator; range; refresh; missing; version	Claims lag and coverage required	typed series, units, bounds, annotations, evidenceRefs

Payer estimated cost

Grain: aggregate model | Window: scenario | Data: low, midpoint, high PMPM

FABRICATED DATA EXAMPLE

Modeled PMPM range - not observed savings



All assumptions, model versions and approvals travel with this chart.

Chart	Required metadata	Decision rule	API output
Interval chart	cohort or person; numerator; denominator; range; refresh; missing; version	Model status, assumptions and approval required	typed series, units, bounds, annotations, evidenceRefs

SECTION 30

Presentation and platform architecture

The GUI reads versioned projections. It does not assemble domain meaning from raw tables in the browser.

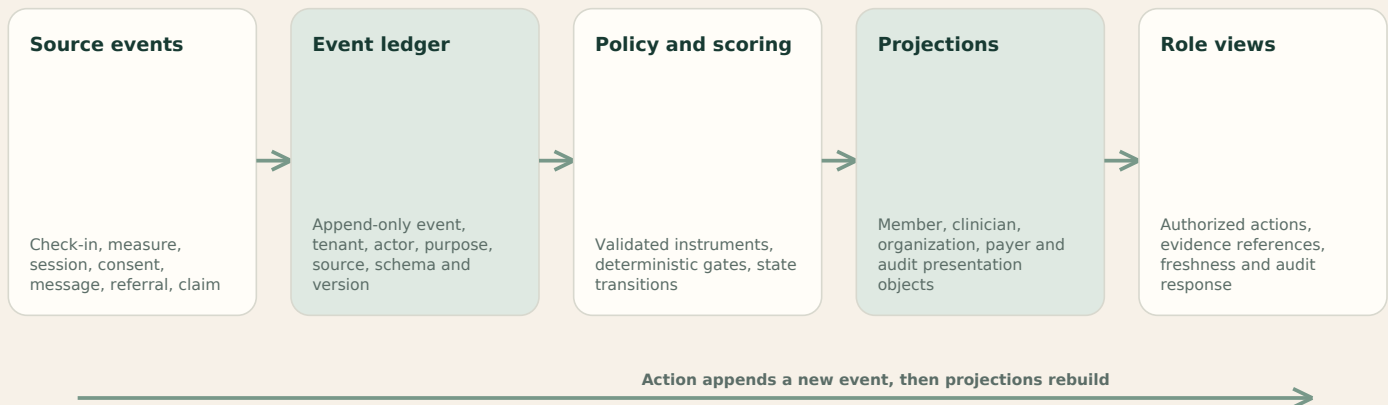
DESIGN RULE

Design from the decision backward. Build from the evidence forward.

30.1 Event, policy and projection pipeline

Write path

Read path



Target write rule

A user action or integration appends a validated event. Domain services update deterministic state. Projection workers build role-specific presentation objects. The browser receives only the projection and actions authorized for that actor.

30.2 Canonical data domains

	Domain	Representation
Identity	Person, account, patient, clinician, organization, payer, care team	Scope, display identity, owner and tenancy
State	Daily state, symptom, function, engagement, context	Current state and change
Measurement	Instrument, version, assessment, score, subscore, severity	Validated trend with bounds and completion
Care	Need, goal, plan, intervention, assignment, response, handoff	Plan state, next action and accountability
Safety	Rule, version, gate result, support, response, re-entry	Authoritative visible state
AI	Observation, draft, evidence, model, version, review status	Bounded model content with status
Enterprise	Tenant, contract, cohort, location, feed, metric, export	Aggregate organization and payer views
Evidence	Source, citation, provenance, correction, audit event	Evidence drawer and replay

Identifiers

- personId is not accountId. A person may have multiple accounts or care relationships.
- Every event and projection includes tenantId and schemaVersion.
- Clinical measures reference instrumentVersion. Gate events reference ruleVersion.
- Every modeled statement references modelVersion, prompt/policy version, evidence and review status.

30.3 Presentation projection services

Page shells

Member, clinician, organization, payer, review and public navigation

Decision components

StateHeader, ChangeSummary, ActionPanel, OwnerDue, GateCard, EvidenceDrawer

Data components

MeasureTrend, SessionResponse, AccessFunnel, CohortChart, DataContractStrip

Presentation service

Role-aware projection DTOs, stale and partial states, action policies

Domain services

Events, measures, policy, consent, identity, tenancy, AI gateway and audit

Pages compose stable contracts. They do not read domain tables directly.

Page name	Domain	Authorized user	Authorized action
member_today	person + active episode	Member web and mobile	check-in, plan, gate, assignment, message
clinician_queue	clinician + tenant	Clinician and care manager	any actionable person change
clinician_patient	person + episode	Authorized care team	measure, state, session, plan, safety, action
organization_overview	tenant + cohort + period	Organization leaders	aggregate event or feed refresh
payer_overview	contract + cohort + period	Payer reviewer	eligibility, claims, metric refresh
evidence_bundle	claim or projection field	Authorized evidence viewer	source, correction or version change
audit_trace	tenant + subject + period	Review and security	every governed event

30.4 Suggested API boundary

GET	/api/v1/member/today	MemberTodayProjection	No domain table joins in client
POST	/api/v1/member/check-ins	Accepted event and new gate state	Safety evaluation commits before success response
GET	/api/v1/clinician/work	ClinicianQueueProjection	Server sortKey and reason codes
GET	/api/v1/clinician/patients/:id	ClinicianPatientProjection	Consent and minimum-necessary enforced
GET	/api/v1/evidence/:ref	EvidenceBundle	Separate permission check and audit
POST	/api/v1/actions/:actionType	Committed action event and projection version	Idempotency key required
GET	/api/v1/organization/overview	OrganizationOverviewProjection	Aggregate and suppressed
GET	/api/v1/payer/overview	PayerOverviewProjection	Contract, cohort and lag included
POST	/api/v1/exports	Export job	Filter hash, purpose and audit event

High-impact action response

The server returns committedEventId, projectionVersion, auditEventId, resultingState and any required follow-up. The client does not show completion before commit.

30.5 Shared component contracts

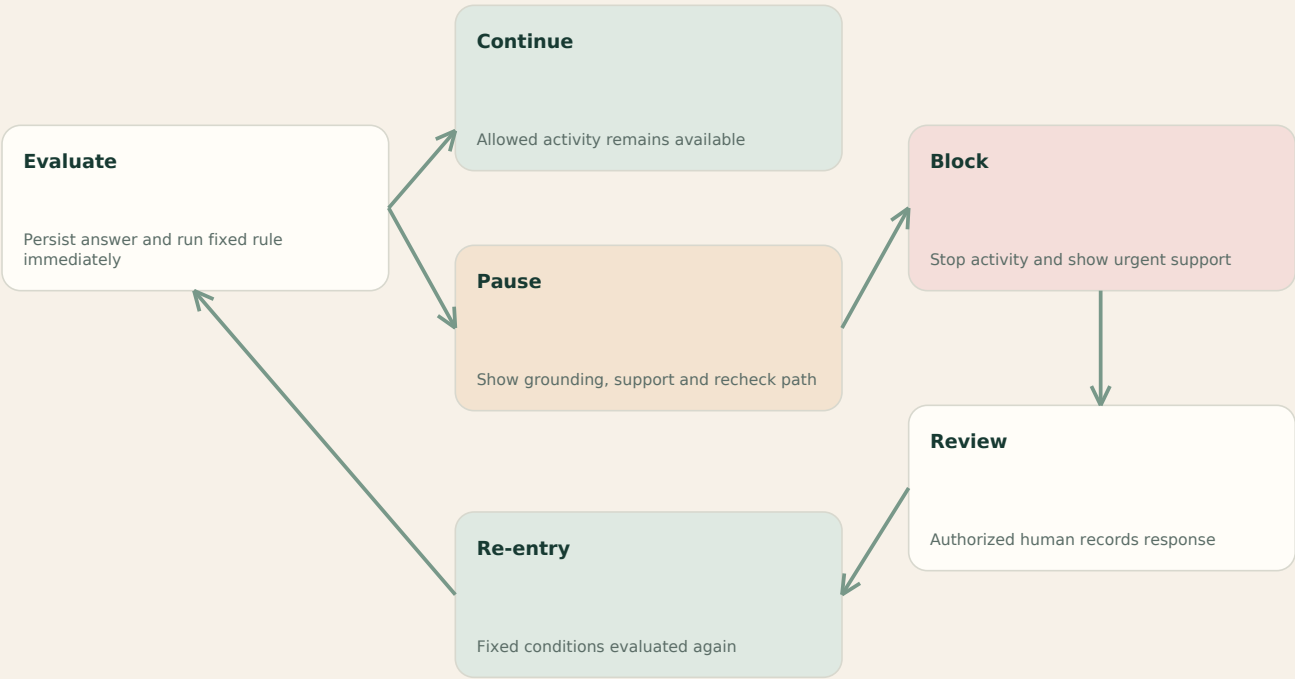
Component	Properties	Notes
StateHeader	identity, state, owner, freshness, access	Shows current state without interpreting missing values
ChangeSummary	items, comparisonAnchor, evidenceRefs	Maximum three decision-relevant changes
ActionPanel	allowedActions, owner, dueAt, confirmation	One primary action; server-authorized only
GateCard	gateState, reason, policyVersion, alternatives, reEntry	Fixed authority and safe alternative
EvidenceDrawer	claim, sources, processing, versions, corrections	Permission check on open
MeasureTrend	instrument, bounds, points, missing, annotations	Instrument-specific scale and accessible summary
SessionResponse	before, close, gate, missing	No outcome overclaim
CohortChart	metric, numerator, denominator, cohort, period, missing	Aggregate only and suppression-aware
DataContractStrip	source, range, refresh, version, exclusions	Always visible under analytic chart
AuditTimeline	events, actors, versions, corrections	Append-only order and replay link

30.6 Permission and consent sequence

		Failure Response
1	Resolve authenticated account and active tenant	Return generic denied state
2	Confirm role and purpose for route	Do not reveal protected subject existence
3	Resolve person-to-care relationship	No routine person view without relationship
4	Confirm current, non-revoked consent and scope	Return consent-required projection
5	Apply minimum-necessary field policy	Remove fields before serialization
6	Apply small-cell suppression for aggregate roles	Suppress value and related derivations
7	Record access audit event	Fail closed for protected evidence if audit write fails
8	Return signed projection version and allowed actions	Client renders only provided actions

**Provider access requires verification plus explicit, revocable consent.
Organization and payer aggregate access does not create person-level care access.**

30.7 Deterministic gate architecture



AI may explain a result. AI cannot make, clear, reverse or override the gate.

continue	Allowed next activity and support	Gate clear with time and policy	gate_evaluated
pause	Reason, grounding, support, recheck	Open review if policy requires	gate_paused
block	Urgent support, care team, no activity	Rule, response due, owner	gate_blocked
responded	Member support path remains	Human response attached	safety_response_documented
re-entry	New check with stated conditions	New result, old result retained	gate_reentry_evaluated

30.8 Stale, partial and failed presentation states

State	Required display	Required action
Loading	Stable skeleton matching final hierarchy	No destructive action
Empty	Explain what is absent and whether that is expected	Offer setup action only if authorized
Stale	Last good value plus exact refresh time and stale reason	Block decisions that require current data
Partial	Show present values and list missing sources	Do not calculate a clean total from incomplete inputs
Permission denied	Generic scope message	No subject or field existence leak
Projection failed	Safe error and retry; retain crisis resources	No fallback to raw domain table
Policy unavailable	Block new session start; keep grounding and crisis paths	Fail closed for safety decision
Audit unavailable	Block protected evidence and high-impact actions	Record operational alert

SECTION 31

Implementation sequence and coding plan

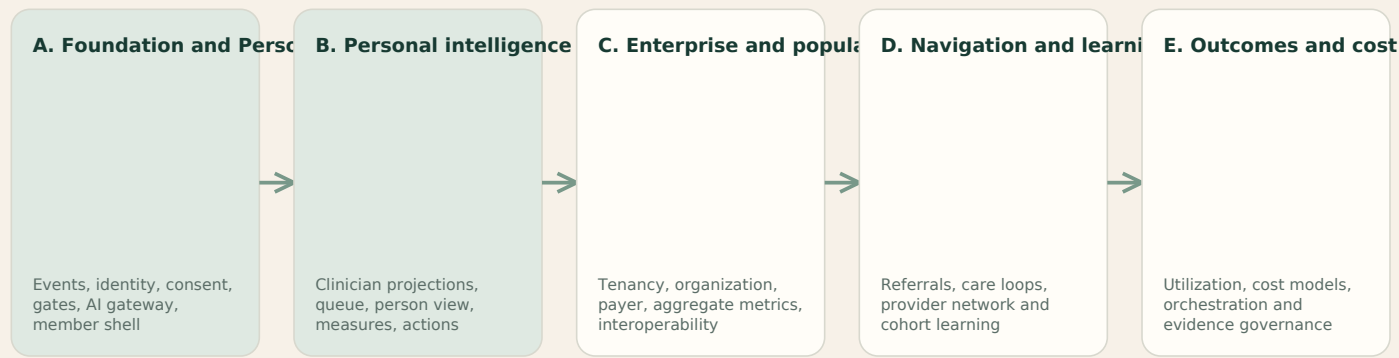
The page layer should arrive in vertical slices that include event, projection, API, component, page, audit and tests together.

DESIGN RULE

Design from the decision backward. Build from the evidence forward.

31.1 A-to-E integration

Design from E backward. Build from A forward.



No phase may break a downstream contract without a versioned migration plan.

The expanded GUI work begins in A and B, but the contracts reserve organization, payer, outcomes, utilization and cost requirements from C through E.

31.2 Recommended coding waves

		Code completion
0 - Baseline	Confirm head, route map, test baseline, data reset and demo identity scan	No personal-looking demo identity; all baseline tests pass
1 - Presentation spine	Projection envelope, stale/partial states, evidence refs, action response, design tokens	One member and one clinician vertical slice work end to end
2 - Member	Welcome, consent, screening, Today, check-in, activities, prepare, session, gate, close, progress	All 15 member screens and failure states pass
3 - Clinician	Queue, caseload, patient projection, safety review, measures, sessions, plan, messages, referrals and audit	All 14 clinician screens and action audits pass
4 - Aggregate	Organization and payer projections, cohort registry, suppression, charts and exports	All 19 aggregate screens pass denominator and privacy tests
5 - Review and public	Review console, lineage, release gates, public product previews and controlled access	All review/public/access states pass
6 - Hardening	Performance, accessibility, security, telemetry, export parity and disaster behavior	Release criteria and clinical/security sign-offs complete

31.3 Vertical-slice definition of done

- Domain event schema and migration are versioned.
- Projection builder handles new, corrected, stale, partial and replayed inputs.
- API enforces tenant, role, consent, purpose and minimum necessary.
- Page renders loading, ready, empty, stale, partial, denied and failed states.
- All actions use idempotency and return committed event plus projection version.
- Evidence references open to sources, versions and corrections.
- Keyboard, screen reader, zoom, contrast and reduced-motion checks pass.
- Unit, projection, policy, API, component, end-to-end and audit tests pass.
- Telemetry proves the screen can be used without capturing sensitive free text.
- Fabricated seed data covers ordinary, urgent, stale, missing, corrected and permission-denied cases.

Do not close a screen ticket because the page renders. Close it when the complete decision path, evidence path, failure path and audit path pass.

31.4 Test matrix

Category	Required tests
Events	schema validation; idempotency; ordering; correction; replay; tenant partition
Measures	instrument version; incomplete scoring; valid ranges; change windows; corrected answer
Safety	precedence; immediate evaluation; pause; block; re-entry; service failure; no AI override
Projection	new event; duplicate collapse; stale; partial; missing source; rebuild parity; supersession
Authorization	tenant attacks; role scope; revoked consent; expired access; purpose; minimum necessary
Clinician queue	stable order; owner; due date; evidence time; duplicate retention; no hidden safety
Charts	denominator; bounds; missingness; small-cell suppression; keyboard legend; accessible summary
Actions	confirmation; no optimistic high-impact completion; idempotency; audit response; retry
Exports	filter parity; cohort version; suppression; purpose; audit event; signed file
Pages	loading; ready; empty; stale; partial; denied; not found; failed; responsive
Accessibility	tab order; focus; labels; 200% zoom; contrast; reduced motion; touch targets
Security	RLS; object scope; evidence reference authorization; no cache bleed; no existence leak

31.5 Acceptance requirements

Stakeholder	Acceptance requirements
Member	Can identify the next safe action within one screen without reading raw scores or system language.
Clinician	Can identify who needs attention, why, freshness, owner and next action from the first queue view.
Clinician	Can open every patient-summary claim to cited evidence and audit history.
Organization	Can identify access or capacity gaps with denominator, period, owner and next action.
Payer	Can distinguish observed results, pending evidence and modeled estimates without interpretation from sales copy.
Reviewer	Can replay a fixed gate and trace a visible statement through event, policy, projection and page.
Security	Can prove tenant, role, consent, purpose and minimum-necessary enforcement for every protected endpoint.
Engineering	Can rebuild all projections from the event ledger and match live output byte-for-byte for the same version.
Product	Can reset and verify fabricated data before every external demonstration.

31.6 Release gates

Gate	Owner	Activities	Blocking condition
Demo identity	Admin and security	Identity scan and reset report	Any personal-looking or historical user data remains
Safety regression	Clinical safety and quality	All policy and failure scenarios	Any gate, support or re-entry failure
Clinical language	Clinical reviewer	Approved page and gate copy	Unsupported diagnosis, readiness or care claim
Projection parity	Data engineering	Ledger rebuild compared with live projections	Any unexplained mismatch
Authorization	Security	Tenant, role, consent and evidence attack suite	Any cross-scope access
Accessibility	Product and QA	Automated and manual checks	Any blocked keyboard or screen-reader path
Analytics integrity	Data and product	Denominator, missingness, suppression, source and version tests	Any clean chart hiding incomplete data
Claims discipline	Product and legal review	Public claim registry	Any target or model presented as demonstrated fact

31.7 Telemetry and operational review

Event	Purpose	Privacy rule
decision_surface_viewed	Measure screen reach and load state	No patient free text
primary_action_selected	Measure whether hierarchy works	Action code only
evidence_opened	Measure trust and review depth	Evidence type, not content
queue_item_resolved	Measure time to accountable action	Reason code, owner role and duration
gate_support_selected	Verify support paths are used	Support option code only
chart_range_changed	Measure analytic use	Range and metric IDs
export_requested	Track purpose and filter parity	Purpose, cohort version and filter hash
projection_stale_shown	Detect feed and projection delay	Projection type, age and source status
permission_denied	Detect access friction and attacks	Actor role and policy code; no subject identity

Operational review should ask whether users reached the right action, whether evidence was available, whether the data was current and whether any role saw more information than needed.

31.8 Final coding rules

- Use one governed evidence spine across all products.
- Build separate role projections. Do not filter a clinician DTO in the browser to create an organization or payer page.
- Keep priority, reason, owner, freshness and allowed actions server-owned.
- Keep deterministic safety decisions separate from model content in code, API and presentation.
- Do not hide stale, missing, rejected or suppressed data behind a clean chart.
- Do not show a percentage without its numerator and denominator.
- Do not show model text without status, evidence and model version.
- Do not make high-impact actions optimistic.
- Do not erase corrections, gate history or review history.
- Do not allow raw event tables to become the default user interface.
- Do not release a fabricated demonstration until identity, history and audit data are reset and checked.
- Design from E backward. Build from A forward.

The finished product should feel calm because it is clear, not because it hides uncertainty or urgency.